PART SIX:

BUT IT DID SAVE LIVES, RIGHT?

The story changed as the evidence emerged. The promise of preventing transmission gave way to the new claim that the real purpose was to prevent severe disease and death. This became the final defence of the vaccine narrative – the idea that, whatever else had failed, lives had been saved and we should all be grateful.

Part six puts that claim to the test. We begin with a closer look at the supposed evidence that vaccination prevented serious illness and hospitalisation. Then, before turning to deaths, we ask a fundamental question – what was counted as a covid death, and how might changes in other factors have reduced that number? We then examine how pessimistic modelling and circular reasoning kept the message alive, and finally, how the medical literature itself sustained the illusion through gatekeeping, bias and distortion.

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PREVENTING SEVERE DISEASE

Failure is only the end if you decide to stop.
Richard Branson 2015[1059]

Once it could no longer be claimed that infections and transmission were affected by vaccination, the narrative changed. The vaccine saviour narrative itself needed saving. Now the claim was that the vaccines were always only ever about preventing hospitalisations and deaths. Changing the goal posts, in this blatant, egregious and unscientific manner was the only way to save the reputation of the vaccines. It worked. The narrative switched from vaccinations stopping covid infections to preventing severe disease and deaths. Stopping hospitalisations and deaths had been part of the sales pitch from early on. Doctors advising Biden wrote on 28 February 2021,

> *The vaccines were all 100 per cent effective in the vaccine trials in stopping hospitalisations and death.*[1060]

Fauci echoed this saying,

> *no hospitalisations and no deaths…These are three highly efficacious vaccines.*[1061]

However, it became the entire sales pitch. Lord Bethel, the UK health minister responsible for authorising the covid vaccines, said in June 2021,

> *The whole purpose of vaccination is to break the link between cases and hospitalisations and deaths.*[1062]

The idea that herd immunity should be the goal was dropped. However, the pharmaceutical companies claimed only that their trials reduced symptomatic infections and we have already seen how that claim was flawed. The regulators authorised the vaccines on the basis that they would reduce infections – there was no other claim for which there was evidence to authorise them. This pivot towards preventing only severe disease and death happened because of a clear failure to prevent infections yet at the same time as it was articulated mandates were being forced on people.

Post rollout studies failed to account for all the differences between the vaccinated and unvaccinated. The imminently dying were disproportionately unvaccinated which led to the clearly biased claim that vaccinations prevented 80 percent or more of covid deaths. In April 2021, the UK government admitted,

> *mortality appears to remain high for people in high-risk vaccination tiers who are admitted to hospital with SARS-CoV-2 infection despite vaccination 21 days or more previously.*[1063]

This message was drowned out as hypothetical modelled data of success was amplified.

The claim that we were in a *"pandemic of the unvaccinated"* was fed by tales of the hospitalised being largely unvaccinated. ITV's Dr Hilary Jones said on breakfast TV in January 2022,

> *Those people who haven't been vaccinated, we'd really love you to think again and be vaccinated because 90% of people in hospital are unvaccinated right now with Covid.*[1064]

The truth was the opposite not least because the demographic who remained unvaccinated was predominantly young and fit and at minimal risk from covid. However, the overall number is not the point. The key is the difference in risk for people of a similar age and risk profile.

SUMMER 2021 HOSPITALISATIONS AND DEATHS

The strongest argument that can be made in defence of the vaccine narrative is that the median age of covid hospitalisations and deaths did fall in the second half of 2021. The trouble is we do not have anything to compare a summer respiratory virus wave to because they have not been measured. For influenza the hospitalisation and death waves were of similar size each winter but we have absolutely no idea how much the number of underlying cases changed from winter to summer nor the age groups affected.

Was this age difference a vaccine benefit, whereby the old were now protected? I am not so sure. Two other possibilities may explain it. The old had far more of their covid cases, hospitalisations and deaths in winter than in summer.[1065, 1066] Consequently, the proportion of hospitalisations that were young were higher in summer and this was true in 2020 as well as 2021. The big difference in summer 2021 was that the young were being injected. This meant they were passing through the two week danger window and its associated immune compromise, putting them at much higher risk than before of infections, hospitalisations and death as discussed earlier.

The UK government finally reported on higher mortality rates in the two week period after vaccination in April 2021.[1067] The mortality rates were more than double the unvaccinated rates. They claimed this may have been due to a bias saying,

> *We have not yet performed analyses accounting for other patient characteristics such as sex and ethnicity, so vaccinated patients may be systematically different (and frailer) from vaccinated* [sic] *patients – this is particularly true of "Tier 2" which is likely to contain many Tier 1 patients who we were unable to identify in our dataset.*

They did not comment on the higher mortality in the first three weeks ignoring the fact the mortality was higher in every age group including younger age groups that excluded the high risk who had been prioritised earlier. It was also true among those at high risk in Tier 6 with a massive 38 percent mortality for those hospitalised in the three weeks after injection. The increase in rates in mortality in the first three weeks after vaccination is striking at every age (see table 5).

The deaths of young people during the danger period of immune compromise in the first two weeks after injection was real and needs to be accounted for. For example, Peter Rossiter, a 39 year old pianist and teacher, developed covid four days after his second dose of Pfizer. His mother described how,

> *his white blood count was almost zero… he very sadly fell very seriously ill very quickly.*[1068]

He died a month after the dose. Pfizer said, *"He wasn't fully protected"*.

Deaths among hospitalised covid patients included in the study		
Priority "Tier" for order of rollout	**Unvaccinated %**	**Less than 21 days from vaccination to symptoms %**
Tier 1 – care home residents and carers	No data as by age only	No data as by age only
Tier 2 - Over 80	19.9	25.5
Tier 3 - 75-79 yr olds	21.2	26.1
Tier 4 -70-74	21.5	25.3
Tier 5 - 65-69	20.8	23.2
Tier 6 - 16-64 in an at risk group	22.6	38.1
Tier 7 - 60-64	21.8	31.5
Tier 8 - 55-59	22.0	30.8
Tier 9 - 50-54	20.9	29.0
Tier 10 - Under 50	21.3	26.9

Table 5: SAGE estimates of percentage of covid hospitalised patients dying by vaccination status in April 2021.[1069]

The US military saw 24 deaths attributed to covid in the 15 months up to mid-2021, before mandating vaccination, but 52 deaths in the last 6 months of 2021 after vaccination rollout. [1070] They claimed 51 of these deaths were in unvaccinated personnel but this seems hard to believe. If they were unvaccinated deaths,

why was the mortality in this small fraction of the population so many times higher than in the unvaccinated population in 2020? Did they repeat the trick of classifying newly vaccinated as unvaccinated?

When comparing covid hospitalisations in children, as we saw, 12-15 year olds had three times as many admissions during their vaccine rollout, than the 2020 pre-vaccination period, whereas for younger children it was only double.[1071] Vaccinated children had a vastly disproportionate risk of hospital admission in summer 2021, including those who were not considered at risk.[1072]

Such deaths and hospitalisations were included in the assessment of how deadly the Delta variant was. Worse these deaths were used as a baseline to model how many deaths would have occurred in older age groups. It was not logical to extrapolate like this when deaths in the younger age groups were higher during their vaccine rollout than at any other time.

When UKHSA extrapolated from this distorted baseline they claimed only 14,000 deaths were prevented in the first six months, by June 2021.[1073] Then, somehow, over the summertime, as young people were injected more and the baseline became more distorted, they claimed they magically saved 123,000 by September.[1074] To put this into perspective, they were claiming that more lives had been saved than the total that had been admitted to hospital in any six month period prior to vaccination. In that period, from 10 June to end of September 2021, there were over 75,000 covid labelled hospital admissions which was three times the year before. Instead of appreciating that this resulted from the immune risk caused by the vaccines, such evidence was waved away on the basis that PCR test results were higher so of course there were more hospitalisations. Taking a step back, it was clear the same proportion was susceptible and the same proportion developed

antibodies. The 'cases' by PCR were inflated. The claims of vaccine success depend entirely on a belief that summer 2021 would have been catastrophic without them.

The Delta wave was flat in the UK, Portugal and Ireland for hospitalisations compared to elsewhere in Europe. These were the three countries that had seen the disproportionate spike in deaths in January 2021 at the beginning of the vaccine rollout. Remember, the overall covid attributed deaths was 13 percent higher than the rest of Europe's from autumn 2020 to spring 2021. If the frail have died more in the winter, they cannot die again in the summer with Delta. If those who would have died in the Delta wave were already dead then this would have caused a survivorship bias.

As discussed earlier, the best an injected vaccine could hope to achieve would be to reduce the length of time that the virus caused immune delay and unhindered replication. Such shortening of this delay might impact on the time to hospitalisation. On a population level the time to hospitalisation remained the same before and after vaccine rollout. The UK government issued detailed reports on the time to hospitalisation by age in 2020, but did not publish this critical data in 2021. A team at Imperial admitted there was no change in the first half of 2021 but presented the data for the second half of 2021 having combined it with early 2022 data such that conclusions about that crucial period cannot be drawn.[1075]

UKHSA published hospitalisations and death rates per 100,000 population for the unvaccinated and vaccinated in summer 2021 which showed more of a commitment to transparency than some other public health bodies. The data showed a worsening trend in the case rate over time for the vaccinated but there was an apparent sustained benefit for hospitalisations and deaths. However, testing differences and underestimating the size of the unvaccinated

population created bias in this data – both in the numerator and denominator of the fraction. We will discuss how much this could negate the difference in *Chapter 30*.

I am very conscious that I am using a variety of different arguments to take down the argument that the vaccines reduced hospitalisations and deaths during the Delta wave. This is exactly the strategy used by believers in the covid consensus to dismiss much of the real world evidence. During the summer of 2021, I believed it was possible that the vaccines had some limited benefit regarding hospitalisations and deaths for the Delta wave. Even now, I admit there is uncertainty. Evidence is often like that. There is ambiguity. Let us say, the vaccine did delay covid by 6 months for some. What then? After the effect had waned those individuals who had survived would be 6 months older and *more* at risk of a covid death. If covid could be paused for six months across the entire population, the overall mortality rate would not rise because the population age distribution would be much the same thanks to ageing, births and deaths balancing each other out. However, for specific individuals who aged during that pause, their excess deaths from covid would be 5 percent higher due to increased age and vulnerability. Furthermore, even if deaths had been prevented, a period of benefit at a population level for a few weeks would not have justified the ethical breaches.

THE HOSPITAL FATALITY RATE

There are two ways a vaccine could have saved lives. It could reduce infections or it could reduce hospitalisations and deaths in those with infections. What happened to the fatality rate in hospitals in the vaccinated and unvaccinated? This is an important measure because there may be less testing bias in hospitals than the community and there is an accurate measure of the size of the population being studied.

Data on death rates in the hospitalised showed no or minimal difference between the unvaccinated and the vaccinated in New York and in England.[1076,1077] Data from Spain showed no significant difference in complications, need for ventilatory support or duration of stay by vaccination status.[1078] In a small study from Ohio, there was an unusual situation where the non-covid mortality did not differ between vaccinated and unvaccinated individuals admitted to hospital, which allows for assessment of covid risk without any statistical adjustments. The covid mortality rate was significantly higher in the vaccinated group and this difference remained significant even after adjusting for comorbidities. [1079] The authors referenced several other papers which also showed no difference in outcomes for the critically ill. Intensive care admissions and death among hospitalised covid patients were also the same in the vaccinated as the unvaccinated in 250 US hospitals. [1080] The authors of this paper nevertheless managed to conclude the vaccine was working by speculating that the vaccinated, *"may be more vulnerable to severe infection at baseline"* i.e. it would have been so much worse without the vaccines.

In England, from the end of May 2020 to the beginning of October 2020, nearly 90 percent of people hospitalised with a covid diagnosis survived.[1081] That figure remained the same from April 2021 onwards until Omicron, indicating no impact of vaccines. It was during the lockdown spikes that the rate was oddly high. SAGE admitted in April 2021, that when vaccinated people were admitted with covid their hospital fatality rate was similar, if not higher, than it had been for the same age group in the unvaccinated.[1082] A separate UK government analysis said,

> *Illness severity at hospital admission was similar across the vaccine groups. This most likely reflects the threshold of illness required for hospital admission.*[1083]

OVERALL COVID HOSPITALISATIONS

Think of a funnel with many infections leading to fewer hospitalisations and then fewer deaths. If there was no impact on total cases and no impact on hospital mortality that leaves a reduction in hospital admissions as the only even-handed way vaccines could prevent death.

Was there a reduction in hospitalisations? US hospitalisations were comparable to previous waves whereas UK hospitalisations had a lower peak in late 2021 (see figure 8). However, there had been higher hospitalisations and more deaths during the January 2021 vaccine rollout in the UK than USA. If the frail were already dead they could not be taking up hospital beds.

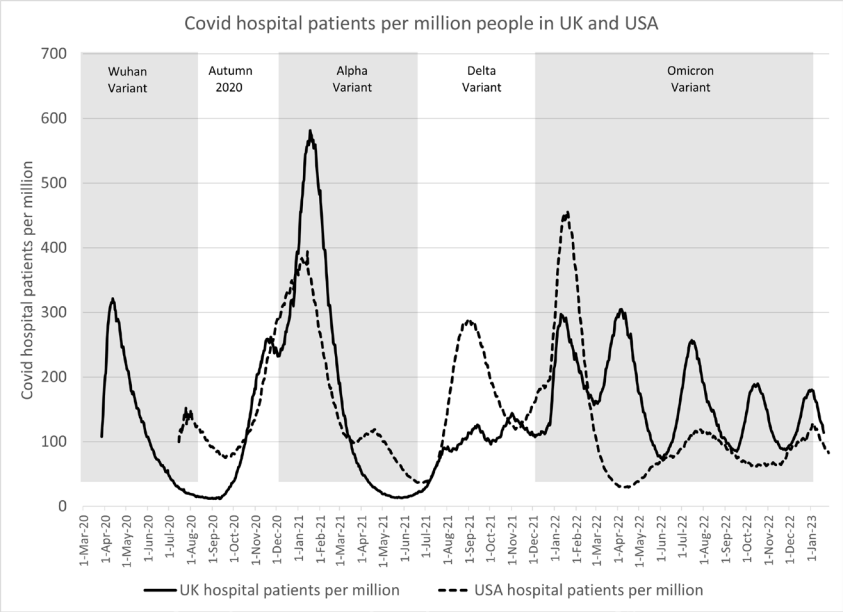

Figure 8: Covid attributed hospitalisations in USA and UK over time

As with other definitions, the definition of a covid hospitalisation changed over time. The earliest definition included any patients admitted for another reason with a positive test result. Once there was pressure to show the vaccines were working, the goalposts were changed. Where it was obvious that a patient had tested positive but could not have covid, these were no longer classified as covid hospitalisations the way they had been. Next, UKHSA introduced a stricter definition of a covid hospitalisation requiring a two day stay and a respiratory diagnosis.[1084] This alone would make the vaccines look effective because the period without vaccines had a broader definition of covid which would inevitably lead to higher covid hospitalisation rates than the period with vaccines simply because of how a hospitalisation was classified. It was like counting every fire alarm as a fire before then installing fireproofing, then quietly redefining 'fire' to exclude smoke and claiming the fireproofing stopped the fires.

The policy around how community pneumonia was managed without antibiotics did not change though. US hospitalisation figures continued to track the levels of virus produced by the population as a whole, as measured, in the sewage using PCR, just as they had before (see figure 9 on next page).

While PCR has its limitations, here it is simply tracking trends over time and any errors would be expected to remain constant. It is a good way of measuring the overall infection rates in the population as a whole without being dependent on people coming forward for testing. It is also not affected by varying numbers of asymptomatic positive results.

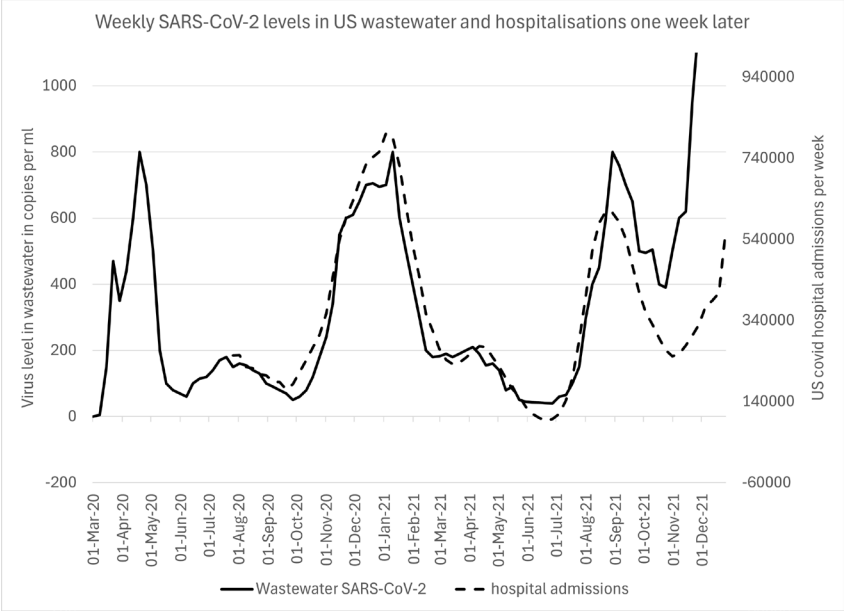

Figure 9: USA weekly hospitalisations (right axis) plotted against virus in wastewater (left axis). N.B. Spring 2020 data not published for US hospitalisations.

How could the infections and hospitalisations have kept tracking? If the vaccines had prevented infections, then an argument could be made that the tiny fraction of unvaccinated people had a disproportionate wave of infections and hospitalisations coincidentally the same size as the earlier whole population waves. In reality, vaccines did not prevent infections. If vaccines had reduced hospitalisations for those infected after vaccination the same size infection wave from virus in sewage should have resulted in a much smaller hospitalisation wave than earlier. It did not. Ultimately the graph is just what you would expect if vaccines did not reduce hospitalisations at all.

MECHANISMS FOR PREVENTING SEVERE DISEASE

The US data shows that the wave of hospitalisations did separate slightly from the viral wave in the wastewater in autumn 2021. Before Omicron, high risk patients could be successfully treated with a drip containing numerous antibodies targeting the spike protein. These monoclonal antibodies are made in mice and are specific to the shape of the earlier variants. When given early to high-risk, often vaccinated, patients, or to hospitalised patients with no antibodies of their own, about 6 percent of people treated avoided hospitalisation or death.[1085,1086] Most treated patients would not have progressed to severe disease regardless, but for a small high-risk subgroup, the effect was meaningful and the claim was that after treating over 3 million people around 200,000 would have avoided hospital. About one week's worth of peak hospital admissions were prevented.[1087] By autumn 2021, when the deviation was seen, over a quarter of eligible high risk individuals in US received the treatment compared to around an eighth earlier in the year.[1088]

Crucially, these antibodies stopped working with the arrival of Omicron. It was claimed that mutations had changed the shape of the spike protein enough to evade antibody binding. Monoclonal antibodies provided high doses of strongly binding antibodies to spike and no longer worked once spike mutated with Omicron. So why were less abundant vaccine-induced antibodies to that same spike credited with preventing severe disease? Is this the point at which people switch to saying it was the T-cell response that was critical after all, not antibodies?

The monoclonal antibodies at best could minimise viral replication during the period of immune dysregulation such that there would be less cell death. Severe disease resulted from extensive lung disease and the impact of the immune response on the rest of the

body. Monoclonal antibodies bypass the need for a functional T-cell response because they are passively delivered. In contrast, vaccine-induced antibodies require active T-cell help for their production and maturation. With Omicron, however, infection largely shifted to the upper respiratory tract and the immune delay seen in earlier variants which prolonged infection did not occur. As a result, risk was already diminished for most people either with or without treatment. Nevertheless, the claim that the reduction in hospitalisations and deaths was due to vaccine and not virus persisted.

HOSPITALISATIONS FOR ANY CAUSE

A fair analysis needs to consider hospitalisations from all causes not just those labelled as covid hospitalisations. Fear of covid, isolation requirements and Do Not Resuscitate orders combined with healthcare denial had kept overall hospital capacity below normal levels but from spring 2021 admissions rocketed and exceeded even the extra capacity that had been expanded for the Alpha wave in January 2021. It was vaccinated people who were being admitted to hospital at these increased rates. Public Health England admitted the hospitalisation rate for non-covid was higher in the vaccinated in May 2021.[1089,1090] Over the following year they published data which showed the non-covid hospitalisation rate had been five times higher in the vaccinated.[1091] It is not clear how much was due to adverse reactions from the vaccine and how much could have been underdiagnosed covid in the vaccinated. There is evidence underdiagnosis was in fact an issue too. Using a leaked NHS dataset from more than one hospital, one statistician under the pseudonym John Dee showed that being vaccinated did not reduce the risk of death from a respiratory infection, suggesting underdiagnosis may have played a part.[1092] Underdiagnosis in the vaccinated was confirmed when patients who had covid-like illness

were later tested with antibodies. Around two thirds of those who had tested negative with PCR during their illness had gone on to develop antibodies.[1093] PCR can fail in more ways than one.

In the Pfizer/BioNTech trial there were five claimed severe cases in the placebo group after dose two and one in the vaccine group.[1094] However, there were two cases both in the vaccine group of severe *"suspected covid"* who tested negative.[1095] Even if there were fewer episodes of covid-labelled severe illness, it is the overall level of serious illness that matters to people so this apparent reduction in severe covid is only worth considering if it is not eliminated by severe PCR negative illness.

After Omicron things only got worse for the vaccinated. A study conducted between late 2022 and early 2023, with patients carefully matched for health factors, examined hospitalisation rates for both covid and influenza after Omicron emerged, showing worse outcomes for vaccinated individuals.[1096] It found that neither the covid nor influenza vaccines reduced hospitalisations. The study included people who had received the covid vaccine but not the influenza vaccine and vice versa – for instance, about 19 percent of the influenza cohort had the covid vaccine without the influenza vaccine – reducing the likelihood that the results just reflect a bias in who chose to get vaccinated.[1097] If these vaccines worked, vaccinating 80 percent of the population should have meant far fewer than 80 percent of covid hospitalisations were vaccinated – perhaps as low as 30 percent – and similarly fewer influenza-vaccinated people among flu hospitalisations. Instead, the proportion of people hospitalized with 0, 1, 2, or more covid vaccines matched the proportion of people in the population who had taken those vaccines. Also, the split between those admitted for influenza and covid was the same no matter how many of each vaccine type they had received. If either vaccine stopped people from ending up in the hospital, you would notice something – like

a lower rate of hospitalisation for those with three doses versus one dose – but there was no difference at all. The study's authors did not have an answer for why this happened.

There were surrogate markers that showed the vaccinated had a disproportionate hospitalisation rate with Omicron. For example, between February and August 2020, white people in California were under-represented in covid attributed deaths.[1098] They made up 37 percent of the population but accounted for 30 percent of deaths. However, between September 2023 and February 2024, white people, who were more heavily vaccinated, accounted for 60 percent of covid deaths.

In the end, the claim that vaccination prevented severe disease rests on an illusion. Once early deaths are classified honestly, once hospitalisation definitions are applied consistently, and once monoclonal antibody effects are separated from vaccine impact, nothing remains of the story we were told. Leaning on outliers such as the UK, Portugal, and Ireland is not scientific, particularly when those same regions had just experienced disproportionate deaths of the frail only months earlier. Everywhere else, the pattern is uniform: waves of serious illness rose and fell in step with infections exactly as they had before, revealing no meaningful disruption in disease severity. The narrative was not sustained using evidence. If hospitalisations were not reduced, and hospital fatality rates were not reduced, then deaths could not have been reduced either. That is the final refuge of the vaccine saviour argument and it is to that last claim we now turn.

Questions arising:
If the outcomes did not change, what exactly did the vaccines achieve?
Did people stop questioning the claims about severe disease and deaths because unlike with infections the failure was not apparent in their own lives?
If the UKHSA hospitalisation rates were higher in the unvaccinated, why did other evidence not show a benefit?

27

DIAGNOSING DEATHS

It is impossible for a man to learn what he thinks he already knows.
Epictetus, c. 108 AD[1099]

To fairly assess whether vaccines prevented deaths, we must first count them honestly. There were clear waves of excess mortality after 2020. These were due to multiple causes, many of which correlated with virus but were not directly due to virus. The assumption that every excess death was a direct result of covid has distorted both policy and perception.

This chapter examines how covid deaths were defined and recorded, and how those definitions shaped the story that followed. It begins with the question of what constituted a *"covid death"*, and how death diagnosis inflated the numbers and resulted

in misclassification. Finally, it explores the evidence around clotting, stress, and treatment harm. The critical question is how many deaths attributed to covid were really due to familiar and preventable causes.

There have undoubtedly been waves of excess mortality since 2020. There are numerous causes behind this but the virus is only one of them. The narrative that every excess death was a direct result of a novel virus has led to immense harm. Even when the virus was supplanted in 2022 by a cold-like variant, Omicron, public fear and policies remained stuck in an outdated paradigm. Mortality rates plummeted. The official claim is that this was due to vaccines as well as the virus itself evolving into a far milder form. However, critically, there was also a change in fear levels and the corresponding approach to treatment of those testing positive, particularly the most frail.

Yet there remains a real question as to how many of the excess deaths were directly a result of this virus. Or to put the point another way, there is a real question as to whether this virus was a statistical outlier compared to other respiratory viruses in the past. We have seen how the total mortality was similar to other bad influenza seasons in *Chapter 4* and after subtracting policy related deaths it would be lower.

As well as a rise in total mortality, covid attributed mortality was considerably higher than historic levels reached for influenza. It may be that some doctors were too quick to diagnose covid over other causes. Am I saying doctors cannot diagnose a covid death perfectly? Yes, I am. Deaths are diagnosed not on a 'beyond reasonable doubt' basis, but on the balance of probabilities, using the evidence available. On 16 April 2020, Patrick Vallance, Chief Scientific Officer agreed saying,

> *ONS rates are people who've got covid on their death certificate.*

> *It doesn't necessarily mean they were infected because many of them haven't been tested. So we just need to understand the difference.*[1100]

Once testing was widely available, the statistics actually got worse. Looking at the events of the last few years through a PCR-test lens gives a totally different impression to examining what happened using other existing and more established measures of public health, such as people with symptoms of disease. As explored further in *Expired*, the PCR test, in conjunction with endless statistical modelling, was an important foundation of the fear narrative. In my view, arguments based on PCR results are so misleading that they should be ignored if other established public health measurements contradict them e.g. numbers of people who were susceptible, became ill or developed covid antibodies.

PCR testing was treated like an infallible oracle. In reality, all that was needed to test PCR positive was to breathe contaminated air. For hospitalised patients that was almost inevitable and repeated testing maximised their risk of testing positive. The dying are most likely to test positive as they struggle to clear viruses that are insufficient to sicken them. Testing those who die from any cause show 47 percent are positive for respiratory viruses by PCR. [1101] Therefore, high levels of positive test results in the dying do not necessarily mean a high number of people died *because* of the virus. Repeatedly testing the dying inflated covid death numbers. If doctors could not definitively rule out covid as a contributing factor in a death, leaving it off the certificate was challenging – especially if the government had already classified it as a covid death in their data.

Some institutions have started to acknowledge these serious statistical problems that I have been trying to highlight for years. By 2024, the FDA had reverted to the established scientific

processes regarding the reliability of PCR testing saying,

> *PCR-positive results do not necessarily represent actual virus... Additional testing is required to determine whether intact pathogen is still present and if it remains infectious.*[1102]

Testing errors led to over diagnosing the hospitalised, the dying and people with viral debris after an infection as well as the asymptomatic. Interestingly, and as a further confirmation, the people running the clinical trials for covid vaccines knew this and excluded all asymptomatic positives from their analysis. Even with all their hope invested in their new technology they knew they could not stop people breathing in contaminated air.

During the first covid winter for every increase in covid labelled deaths there was a fall in deaths from other causes, for example, as covid deaths rose cancer deaths declined and when covid deaths declined cancer deaths reappeared (see figure 10).

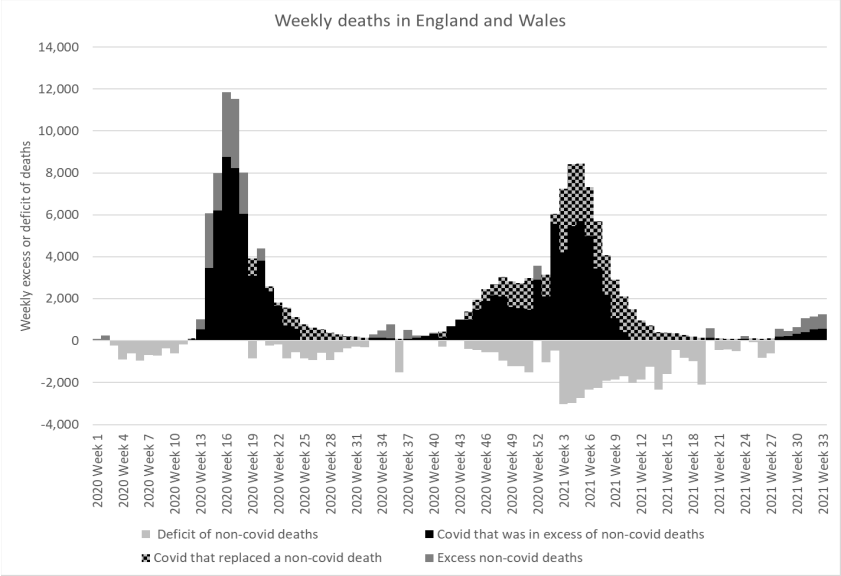

Figure 10: Weekly deaths in England and Wales. Weekly covid labelled deaths (black bars), covid labelled deaths that replaced a non-covid death (checked bars and deaths from all other causes which were lower than expected when below zero and higher when above (grey bars).

This suggests they had been switched. There is no clearer indicator of over diagnosis than that. People dying of other causes were miscategorised as covid deaths creating the illusion that covid was far deadlier than it was. Critical evidence can be found in actuarial analysis, where livelihoods depend on accuracy in these matters.

In a report, the reinsurer Swiss Re said,

> *we find evidence of inconsistency in the causes of death recorded over this period, with signs that other causes of death were misclassified as COVID-19.*[1103]

This miscategorisation was not as bad in other countries and improved in the UK after the first covid winter. However, this is an excellent demonstration of how generous covid death diagnosis was. An increase in deaths from the multiple dangerous and damaging policy interventions would result in both covid and non-covid deaths but the non-covid deaths would be obscured by overdiagnosis of covid deaths.

In the USA, if doctors described a death as a covid death, relatives would be given a $9,000 grant for funeral expenses up until 30 September 2025.[1104] Hospitals were also paid two to three times more in Medicare payments for covid patients than patients with other respiratory illnesses.[1105] It would be naive to think such incentives do not, or did not, distort the data.

Having said all that, we were repeatedly told that *SARS-CoV-2* was not like other respiratory viruses and was causing significant numbers to die from clots.

WHAT CAUSED THE DEATHS FROM CLOTTING?

That first covid winter, mid-lockdown I bumped into a doctor friend on a dog walk. I asked how many of her covid patients had

respiratory symptoms. *"Surprisingly few"*, she replied, while carefully keeping the required distance from me,

> *You have to understand, this virus does so much more than just that. We have lots of heart attacks and strokes, some pulmonary embolisms and even some patients with only gastrointestinal symptoms.*

It struck me that this could describe the typical range of patients arriving at any hospital's front door. What if they were overdiagnosed? Doctors placed utter faith in positive PCR test results, despite some being false positives, making overdiagnosis inevitable. Patients with any condition could test positive in hospital – surrounded by virus-laden air – and be labelled covid cases despite no respiratory symptoms. This raises the question: how much of the claimed clotting was truly caused by covid, and how much simply reflected the usual background of medical emergencies being recast under a new label? Suppose the test had never existed – would we have probed other causes? Could these causes have contributed?

If covid were directly responsible for clotting outside the lungs, we would expect a dramatic surge in 2020 in strokes, heart attacks and deep-vein thromboses. Yet stroke deaths did not increase. Heart-attack deaths and pulmonary embolisms did rise, but this coincided with 200-500 fewer people per day in England calling ambulances for chest pain, leaving thousands untreated. [1106, 1107, 1108] (By contrast, in 2021, after the vaccine rollout, ambulance calls for chest pain, cardiac arrest and stroke rose far above 2020 levels). Hospital admissions with a pulmonary embolism were below expected levels in spring 2020 but rose from autumn 2020.[1109]

For those with severe covid the clotting problem was largely concentrated in the lung capillaries. Post mortem studies did

report tiny clots or micro-thrombi in several organs, but only a couple used a control group to compare to other pneumonias. One comparison with influenza pneumonia found nine times more micro-clots in covid lungs than in flu lungs, though the covid patients were older and sicker. [1110] A more rigorous but small study compared covid deaths with non-covid pneumonia in elderly controls found an excess of micro-clots in the lungs but not in other organs.[1111] Sick patients have always clotted more.

These micro-clots likely contributed to the "ground-glass" appearance often seen on chest X-ray and CT scans. They were important because they blocked parts of the lung circulation and pushed blood towards lung regions with poorer airflow, making oxygen transfer less efficient. Normally, blood vessels in poorly oxygenated parts of the lung squeeze tightly shut to redirect blood to more oxygenated areas. People with diabetes, hypertension, obesity or old age may have blood vessels that do not constrict as effectively, making them more vulnerable to this potentially lethal problem.

Claims that covid could increase clotting risk were supported by evidence from laboratory studies which showed spike protein triggering the formation of unusual clots that are harder for the body to break down. [1112] Some speculated that, spike protein could also bind to ACE-2 receptors on the vessel lining 'endothelial' cells and impair their natural anti-clotting functions.[1113] In both cases the requirement is for circulating spike protein but, as we have discussed, whole virus does not enter the blood.[1114] Even when researchers hunted hard for it, viral RNA was not found in these endothelial cells. [1115] If clotting did result from spike it could only have been from free-floating spike from viral debris, which would explain why it was localised to the lungs.

What was damaging the endothelial cells to enable clot formation? It was not direct viral damage but the immune attack on the neighbouring infected respiratory epithelial cells which created a local flood of inflammatory chemicals that washed across the adjacent endothelium, pushing it into a pro-thrombotic state.[1116,1117] In mild cases, modest cytokine levels may cause only transient, small-scale clotting, whereas in severe cases the far higher inflammatory load can push the endothelium into sustained dysfunction. Prolonged respiratory support can add further stress to the already fragile tissue at the alveolar-capillary border, increasing leakiness at that interface. These indirect processes explain how covid could be fatal in a minority of severe cases without systemic viral damage.

Other factors may have contributed to the higher rates of lung micro-clots in covid compared to influenza pneumonia patients. Inflammation, dehydration and stress all increase clotting risk. Sustained higher levels of inflammation, and the associated clotting tendency, are more likely in undertreated pneumonias compared with pneumonias treated early.

Dehydration increases clotting risk several fold in hospitalised patients specifically.[1118] Many covid patients arrived already dehydrated and their condition often worsened in hospital, whether due to neglect, fluid loss from fever, or the use of dry oxygen. [1119] When dry oxygen is used on the wards, patients can lose up to around a litre of water per day simply through the extra humidity they add to the air they exhale.[1120] Even on mechanical ventilation, where the gas is humidified, the process is imperfect, and patients still lose more water than they would when breathing normally. With visitors banned and catering staff reluctant to enter covid wards, dehydration was often overlooked. Care home evidence backs this up: dehydration worsened mortality in 2020.[1121]

Stress added another layer of risk. Fear messaging, lockdowns, isolation and the dread of being labelled a covid case all produced extreme psychological stress.[1122] For example, bereavement carries risks of medical complications, including cardiac death – the widowhood effect.[1123] Economic downturns, natural disasters and other kinds of social upheaval like the USSR's collapse have historically driven excess cardiovascular mortality through stress-induced changes affecting clotting.[1124,1125] Pre-existing anxiety was the strongest predictor of covid death.[1126] Anxiety comes with a high risk of clotting, with a 50 percent increase in the rate of clots in the deep veins of the leg (DVTs).[1127] There are two broad explanations for this. On the one hand, anxiety and associated over breathing may have been over tested and over treated with interventions that may have been detrimental. Hyperventilation can lower oxygen saturation to levels that early protocols interpreted as grounds for mechanical ventilation.[1128] On the other hand, the stress hormone, cortisol, makes blood more prone to clotting and its lethal consequences.[1129]

Even if spike protein increases clotting risk in laboratory conditions, it does not follow that every clot in a covid-positive patient was caused by the virus. No study has compared the clotting seen in covid patients with that seen in non-covid patients experiencing equivalent inflammation, dehydration and stress. Attributing every clot to the virus simply because a test was positive is an oversimplification that does not withstand scrutiny.

HOW MUCH WERE DEATHS EXAGGERATED?

There are ways to estimate how exaggerated the covid deaths claim is. Fatality rates per antibody positive case, even for the same age groups, varied hugely between countries highlighting overdiagnosis of covid death in the UK.[1130] Furthermore, in that

first winter (2020-2021) covid-labelled deaths in England were three times higher than excess deaths coded as acute respiratory infection – an umbrella term which includes all patients with a covid label and respiratory symptoms.[1131] Even if a fraction of covid deaths were actually from cardiac causes rather than respiratory, the discrepancy here is beyond belief.

To give an estimate of the extent of overdiagnosis we can assume that the virus kills anyone of a particular age at a constant rate. If more deaths are attributed to a virus than expected it is reasonable to think they are due to something else. Over time deaths per hospitalisation have been very stable. Since 2022, almost exactly 8.5 percent of patients in hospital with a positive test died with a covid labelled death.[1132] That has remained constant for every month of the year for three years. Prior to 2022, the figure was higher but it was not constant and there were periods that were marked outliers. For five months, April and early May 2020 and from late October 2020 to end February 2021, a far higher proportion of deaths were attributed to covid. The higher peaks could be either from too few hospitalisations or too many deaths. Even if intensive care departments were full, the general wards were not. The only evidence of people being denied hospital care was for groups at highest risk of dying. Had they been admitted the fatality rate may not have been much lower. If it had been a lack of admissions that caused a higher rate of deaths per admission then there would have been no impact on deaths per case at a whole population level. However, according to the Cambridge University Biostatistics Unit the percentage of covid deaths per antibody positive case – which should be a constant – was far higher in lockdowns than between lockdowns.[1133] That leaves a situation where people were dying at an increasing rate as fear escalated with peak admissions and less so as the peak subsided. It is not reasonable to attribute all hospital "covid" deaths to a virus if the mortality rate was over twice as high during the

panic periods than the periods in between. The higher mortality is suggestive of panic and protocol driven increases in mortality when pressures were high and not of inevitable deaths because of a virus.

All calculations of infection fatality rates are inflated because of flawed death data. If half the deaths labelled as covid were overdiagnosed, then the claimed fatality rate can be halved. Neil Ferguson of Imperial College, who had predicted 500,000 UK covid deaths, stated in summer 2020 that two thirds of those who died would have died within a year regardless.[1134] If that were true there should have been a dramatic death deficit in 2021 which never came. A figure which excluded deaths for those at imminent risk of dying would be lower still. Removing deaths due to interventions that increased mortality would lower the total deaths even more. Deaths that were in the overdiagnosed, imminently dying or from interventions could not be prevented by an injection. Reversing the overdiagnosis and dangerous interventions would create another illusion of vaccine benefit.

Questions arising:
If a PCR label can replace a diagnosis, how can death data be trusted?
Could an inflated infection fatality rate be used to scare people in future?
Will we ever know how many deaths were caused by policy in this environment?

28

IT WOULD HAVE BEEN SO MUCH WORSE

My worst habit is my fear and my destructive rationalizing.
Sylvia Plath, 1950[1135]

The claim that vaccines stopped deaths could not be sustained once it became clear that vaccinated people were continuing to die of covid. To preserve the illusion of success required yet another shift in the narrative from preventing deaths to saving millions of hypothetical lives. It was said that, without them, the outcome would have been catastrophic – *"it would have been so much worse."*

This chapter examines that belief. It begins with the models that created the illusion of mass salvation – the extravagant estimates

of millions of lives saved and the circular assumptions that produced them. It then looks at what the data actually shows – steady global death rates, waves no larger or smaller after vaccination than before, and a pattern that stubbornly refused to fit the models. Finally, it asks how speculative models and wishful assumptions came to replace real-world evidence, turning uncertainty into apparent proof.

The belief it would have been so much worse without vaccines extended to the whole of society. People believed millions of lives were saved. If an audacious claim could be made – like 20 million lives were saved by vaccination – then any resultant harm could be justified by claiming it was worth it for the greater good. (The fact that many of those harmed were never at risk from covid death gets brushed aside).

Imperial College was the go to institution for covid modelling for the UK government but also globally. Neil Ferguson's team published the worst cases scenario predictions that were used to justify lockdown. The past record from Imperial on modelling claims is shocking. In 2001, Neil Ferguson's models led to the culling of 6 million cattle and sheep, allegedly to prevent spread of foot and mouth disease, which cost the UK economy £10bn. Subsequent predictions by Ferguson have included an estimated 50,000-150,000 deaths in humans in 2002 due to CJD because of the BSE outbreak (there were 177 deaths), a predicted 200 million deaths from bird flu in 2005 worldwide (there were 78); and 65,000 deaths from swine flu in the UK in 2009 (there were 457). Although this was not Neil Ferguson's work, members of his team were among the Imperial modellers who produced a claim that 20 million lives had been saved thanks to covid vaccines.[1136]

Modelling forecasts like these are nothing but glorified guessing based on extrapolating from assumptions of the authors' choosing. As we have seen, with the examples above, it is mostly worthless. The assumptions in this case were that the vaccinated would have lower mortality than the unvaccinated and from this they produced a global extrapolation of vaccine impact. They referenced a prior literature review authored by some of the same team along with doom modeller Neil Ferguson, and even used data from biased studies, including trial claims of 100 percent protection against severe disease after AstraZeneca and near that for Pfizer. [1137] They added in their favourite unevidenced assumption that everyone would have been infected in the absence of intervention. The resulting 20 million figure can be considered an absurd fantasy.[1138]

Plotting global covid deaths over time shows a steady line that ascends at a constant rate up until vaccination started. The rate since has continued inexorably (see figure 11 on next page). There was certainly no drop in the rate at which deaths accumulated when vaccines rolled out. The gradient did change with Omicron in 2022. If vaccine had reduced deaths to a third the way Omicron had then the same change of gradient should have been seen on vaccine rollout. The claim was that deaths would be reduced to between 5 and 10 percent of previous levels in the vaccinated so the change should have been even more dramatic.

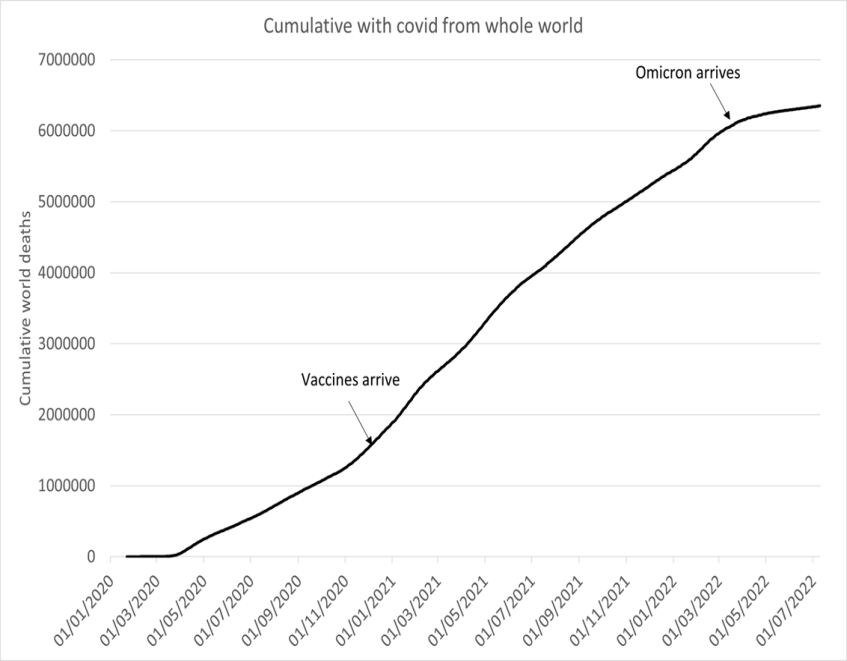

Figure 11: Cumulative global covid attributed deaths

If the modellers were right then, without vaccines, the accumulating covid deaths graph would have had to take a sharp turn at the end of 2020 such that deaths would have massively accelerated to over three times the prior weekly death rate. Only then could enough deaths have occurred to make the claim that the vaccine prevented 20 million deaths. Below is the graph of the predicted outcome without the vaccines (dotted line in figure 12).

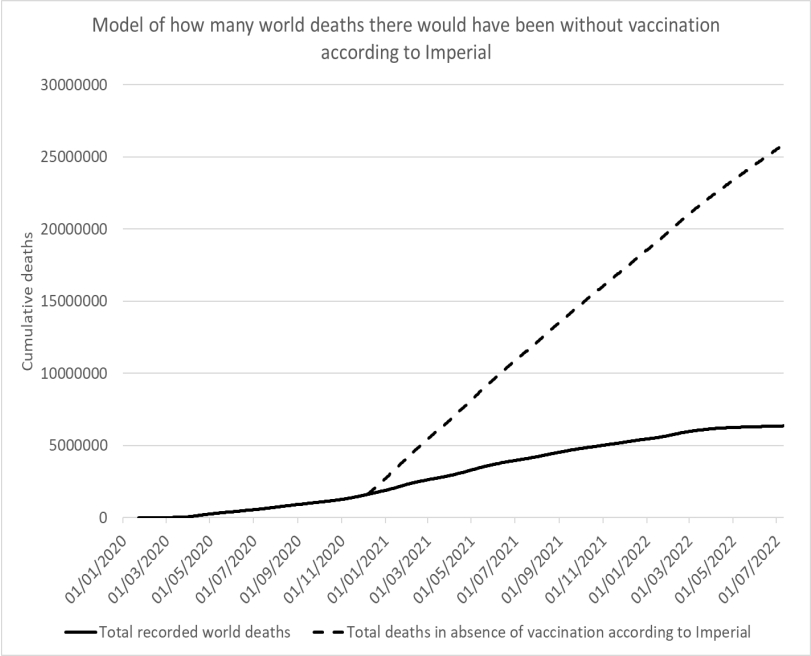

Figure 12: Cumulative global covid attributed deaths (solid line as in figure 11) with claimed deaths in absence of vaccination (dotted line)

To reach 20 million they had to claim that 500,000 lives had been saved in the UK even though that was the worst case scenario prediction if 85 percent had been infected and there had already been 200,000 covid attributed deaths when the paper was published.[1139] For the US, they claimed 1.9 million lives had been saved by the end of 2021 even though the worst case scenario prediction was 2.2 million lives at risk and there had been 820,000 deaths attributed to covid. Globally there had been 6.5 million covid attributed deaths when they claimed 20 million lives had been saved.

The paper had other obvious flaws. They estimated that the same number of doses saved three times as many lives in Hong Kong, 130 per 10,000 vaccines, as they saved in the Netherlands, 42 per 10,000 vaccines.[1140]

The 20 million figure was nothing more than a big number that could have been pulled out of thin air. Big numbers were thrown out repeatedly but with very little agreement. In July 2021, UKHSA claimed 30,000 lives had been saved and within six weeks that claim had risen to over 100,000 lives. [1141, 1142] By the end of the year the DHSC was claiming only thousands of lives had been saved.[1143] The ONS also claimed only thousands of lives were saved worldwide by 2022.[1144] In 2023, the MHRA and a health minister claimed tens of thousands of lives had been saved while the NHS claimed just thousands of lives were saved.[1145,1146,1147] By contrast that same year Donald Trump claimed 100 million lives had been saved.[1148] In 2025, Peter Hotez claimed that mRNA alone had saved 3.2 million US lives, which is 1 million more than the doom modellers predicted were at risk in a worst case scenario.[1149] If scientists carry out separate analysis but with increasing access to data over time, their estimates should converge towards the truth. Here the estimates diverged over time. What does that tell you?

A different *Lancet* article had claimed, using modelling, that over 18 million people had died globally from covid by the end of 2021.[1150] To get to an estimate of between 14 and 20 million lives saved they factored in these 18 million lives lost but assumed that the entire world would have been infected by the end of 2021 and 0.54 percent would die without vaccines. They claimed that 31 to 38 million lives could have been lost by end 2021 in the absence of vaccine. (I am aware those do not add up).

All the assumptions used were unfair. Firstly, the WHO reported 1.8 million covid attributed deaths in 2020 which they rounded up to 3 million.[1151] How could 6 times as many lives be saved the following year when people were still dying? Secondly, the WHO estimated the fatality rate per case to be 0.23 percent, at most –

and that was after assuming every covid attributed death was actually due to a virus.[1152] Even if that mortality rate was true it would take 16.5 billion infections by end 2021 to reach 38 million deaths i.e. everyone in the whole world would have had to be infected at least twice. A US Gallup poll surveyed the population finding half reported ever being infected by 2023 rising to only 70 percent by spring 2024.[1153]

WHY THE 20 MILLION LIVES SAVED MODEL IS WRONG

The waves of deaths across Europe and the USA were roughly the same size before and after vaccines were introduced. This is a simple starting point – looking at the data as it stands. If someone claims that vaccines saved millions of lives, they are suggesting that the death waves after vaccination were smaller than they *would have been* without vaccines. But since the waves remained broadly the same size, that claim relies on imagining a hypothetical scenario – a much worse outcome in 2021 than occurred in 2020.

Now, to imagine that worse scenario, you would need a model predicting that without vaccines, the post-vaccine waves would have been massive – not any old wave but a tsunami – far bigger than what we actually saw anywhere. However, if the virus was capable of causing such enormous waves after vaccines, why didn't it cause larger waves *before* vaccines anywhere, when no one had immunity from shots but virus was still spreading long distance through the air? Why did it not cause larger waves in low vaccination countries? The pre-vaccine waves were not vastly bigger than the post-vaccine ones. Nothing changed.

To believe the pre-vaccine waves were not as huge as they could have been, you would have to assume lockdowns were highly effective – dramatically reducing infections and deaths before vaccines came along.

Some lockdowns were too early and some too late to have been responsible for the peak in cases and consequently there are numerous papers showing they had no effect while the ones that claim the opposite are based on modelling.[1154,1155]

The idea is that without lockdowns, the virus would have infected nearly everyone all at once, like a tsunami, overwhelming hospitals and causing millions of deaths. Lockdowns were sold as being able to slow this inevitable spread, squashing the curve. Yet, somehow, a belief was adopted that they caused infections to peak – suddenly stopping the virus altogether. But then, when restrictions eased, there wasn't the immediate explosion of cases you would expect if lockdowns were the only thing holding it back. Instead, bigger waves came later, in the autumn and winter of 2020 – right on schedule for respiratory viruses, which peak in colder months every single year.[1156]

That first winter peak was sometimes credited to lockdowns keeping things under control and sometimes to vaccines, even though fewer than 2 percent of people had been vaccinated by then. Neither explanation fully fits. If lockdowns were so powerful, why did cases begin and continue surging after their implementation in some places? And if vaccines were the key, how could they take credit when barely anyone had taken them? Nevertheless, people did accept the idea that pre-vaccine waves had been controlled thanks to government interventions. The fact only a fraction of the population was susceptible to any one variant was glossed over and the fact that there continued to be waves long after vaccination with covid deaths continuing was also glossed over. Instead of considering these points, people were led to believe that the post vaccine wave would indeed have been a tsunami in the absence of vaccines and that therefore 20 million lives had been saved.

The belief in the tsunami coupled with the hoped for total protection from vaccines caused Neil Ferguson to claim in July 2021 that by late September,

> *the bulk of the Covid pandemic will be behind us.*[1157]

A third of covid labelled deaths came after this deadline with the reduction due to a return to normal medical care and the Omicron variant. Just as we saw with the lockdown modelling in *Expired,* the modelling of lives saved were like cardboard cutout lives – it was a fantasy. Yet, these cardboard cutout predicted lives saved are wheeled out again and again to justify the harm caused, even after they had started to peel at the edges and crumple, even after they look clearly false.

Of course people want to believe lockdowns saved lives – after all that was sacrificed – but unfortunately there is no evidence to back the claim up. The UK formal pandemic plan was last published in 2014.[1158] Its recommendations were effectively adopted and implemented by Sweden. The UK, despite following its principles initially, dropped the plan abruptly in 2020. These pandemic plans, written by experts in an atmosphere entirely divorced from the covid era of hysteria, panic and fear, called for,

> *supporting the continuation of everyday activities as far as practicable*

and,

> *promoting a return to normality and the restoration of disrupted services at the earliest opportunity.*[1159]

If lockdowns did not work (and could not work because of long distance aerosol spread), then the pre-vaccine waves were already as big as the virus could naturally produce and the post-vaccine waves being similar in size would suggest vaccines did not change anything. (Remember that Portugal, Ireland and the UK were outliers in summer 2021).

DOING A SENSE CHECK

If you go along with the fantasy that the vaccines were as effective as claimed then some simple extrapolations quickly reach impossible figures. To figure out the impact of vaccines, public health agencies took the number of cases that did occur scaled up to the number that they believed would have occurred without their miracle vaccines. For example, Public Health England assumed that 95 percent of the cases had been prevented. When it was assumed that the cases that had in fact occurred represented only a fraction of what would have occurred without vaccine this resulted in a ludicrous claim that a total of 23.4 million cases had been prevented by August 2021 in England alone.[1160] Only 38 million people had been vaccinated. That amounts to claiming that without vaccination, 60 percent of those injected would have been infected within just a few months despite only 11 percent of household contacts being susceptible.

A more extreme claim was made by Dr Eric Topol, an expert in digital medicine who said the death rate in the unvaccinated was 417 times higher than in the vaccinated.[1161] All he had to do was look at the fact there were similar numbers dying as before in a population that was largely vaccinated to see that his claim could not possibly be true.

The claims that it would have been so much worse without the vaccines were heard repeatedly on an individual level. Vaccinated politicians, celebrities and journalists would announce their covid infections on Twitter and include in the announcement the words, *"I am grateful I was vaccinated"* and often *"it could have been so much worse"* or similar. Numerous US politicians including Barack Obama, Bill Clinton, Hilary Clinton, Gavin Newsom and Kamala Harris all repeated the line as did the CEO of Pfizer and numerous

others.[1162,1163,1164] The UK Health Secretary joined in, making a video saying,

> *This morning I tested positive for Covid. I'm waiting for my PCR result, but thankfully I have had my jabs and symptoms are mild.*[1165]

Steve Baker MP, who helped lead the group of conservative MPs opposed to lockdown was not immune to the cult either saying,

> *Positive result just in. Now isolating etc, thankful to AZ that symptoms so v mild.*[1166]

THE VIRUS COULD HAVE BEEN SO MUCH WORSE

The vaccines were not miracles but there were 'covid miracles'. There are reasons to be very grateful at how many aspects of the covid era unfolded. Covid is not a grave danger to children, is not as lethal as it seems on paper, is not able to infect everyone with each wave and there are effective treatments that can save lives. We should be hugely grateful for these facts. Moreover, the evidence shows no benefit from interventions including lockdowns and masking; vaccine passports and mandates; coercion through fear and manipulative marketing and pushing vaccines on the young right down to babies. None of these altered the virus' trajectory and this sharpens the moral distinction between right and wrong on these policies, making it easier to advocate against them with confidence. If either lockdowns or vaccines had shown some tangible benefit, the core ethical objections to them would remain unchanged, but there might have been a murky space of attempted practical justifications to muddy the debate. As it stands, no such justification exists. Given the profound implications these policies could have on our future and that of our children, this clarity is something to be deeply and eternally grateful for.

THE VACCINE WAS SO MUCH WORSE

Unlike the virus, the vaccine did turn out to be worse. For many of the vaccinated the effect on the immune system was bad news. First, for those at more risk of infection in the first two weeks after vaccination the vaccine did make things worse. From 2022, with Omicron, the risk of infection was also higher in the vaccinated.

Up until spring 2022, UKHSA continued to claim a lower covid admission rate for the vaccinated.[1167] After March 2022, the tables reporting a rate per 100,000 stopped being reported. However, from August 2022, the percentage of admissions through the emergency department by vaccine status was included in their regular reports.[1168] From March 2022 onwards, the percentage had remained stubbornly at or below the total percentage of unvaccinated in the population indicating a lower risk for admission in the unvaccinated than the vaccinated before factoring in any bias in the data. After June 2023, although the reports continued, there was no more mention of this figure.[1169]

Despite there being no actual benefit, the extraordinary claim was made that commentary on social media had led to a lack of compliance that resulted in a third of all deaths attributed to covid by January 2023.[1170] The *"it would have been so much worse without the vaccine"* argument was never grounded in reality. It was a convenient shield to deflect criticism, silence dissent and justify the relentless push for booster after booster. But as time passed, the contradictions became glaring and the once-unquestionable claims unravelled.

The implications are staggering. If vaccines did not significantly reduce hospitalisations or deaths and if their rollout coincided with increased covid and non-covid illness, what was the true cost of this unprecedented global medical experiment? Public health

officials abandoned rigorous science in favour of fear-driven policies, manipulation and coercions which left a legacy of harm. The data is clear – it was not the unvaccinated who posed the greatest risk – it was the blind faith in a narrative that never aligned with reality.

Questions arising:
If those in power can always claim *"it would have been so much worse,"* how can a society ever hold them to account?
When models replace measurement, who is left to defend reality?
If hypothetical deaths can justify real harms, what limits remain on state power?

29

WERE ANY LIVES SAVED?

Science is the belief in the ignorance of experts. When someone says 'science teaches such and such', he is using the word incorrectly. Science doesn't teach it; experience teaches it.
Richard P. Feynman, 1999[1171]

The previous chapter examined the models – the fantasy where millions of lives were saved. This one turns to the real world. If those claims were true, the evidence should be unmistakable: deaths should have fallen, excess mortality should have declined, and countries with higher vaccination rates should clearly have outperformed those with lower uptake.

Here, we test that promise against reality. We begin with how deaths changed over time, comparing pre- and post-vaccine waves, then look across countries and regions with contrasting

vaccination rates. We examine how official statistics, particularly those from the ONS, created illusions of benefit through misclassification and bias. Finally, we turn to the places most heavily vaccinated before their first major wave, Australia, New Zealand, South Korea and Hong Kong, to see whether early coverage made any measurable difference.

DEATHS OVER TIME

There were more excess deaths in the second half of 2021 than the second half of 2020 (see table 6 below and table 4, *Chapter 12*, page 156). Given that people can only die once, we would have expected fewer deaths the following year because the number of frail people would be diminished. Finding a higher number of deaths one year after a period of abnormally high mortality is very disturbing.

England government data	**Jan-Jun 2020**	**Jul-Dec 2020**	**Vaccine rollout begins**	**Jan-Jun 2021**	**Jul-Dec 2021**
Covid labelled deaths	48,628	32,276		52,494	17,823
Covid labelled deaths over whole year	80,904			70,317	
Number of overall deaths above 2015-2019 average	52,298	19,379		22,872	29,634

Table 6: Impact on deaths in six-month intervals before and after start of vaccine rollout in England in December 2020

Comparing disease burden and deaths over time is fraught with difficulty because of how testing was carried out. However, if we ignore the testing and measure disease burden based on virus in sewage and compare that to excess mortality over time we can clearly see that each wave was very similar in the USA before and after vaccines (see figure 13). If a vaccine had worked at stopping infections the post vaccine wave of virus in the wastewater should have been negligible. If a vaccine had prevented deaths the level of deaths should have stopped tracking the amount of virus present. It is simply not possible to have an intervention that reduces deaths after an infection by more than 90 percent and not see a dramatic flattening of the death waves compared to the infection wave. That did not happen in USA or Europe as a whole.

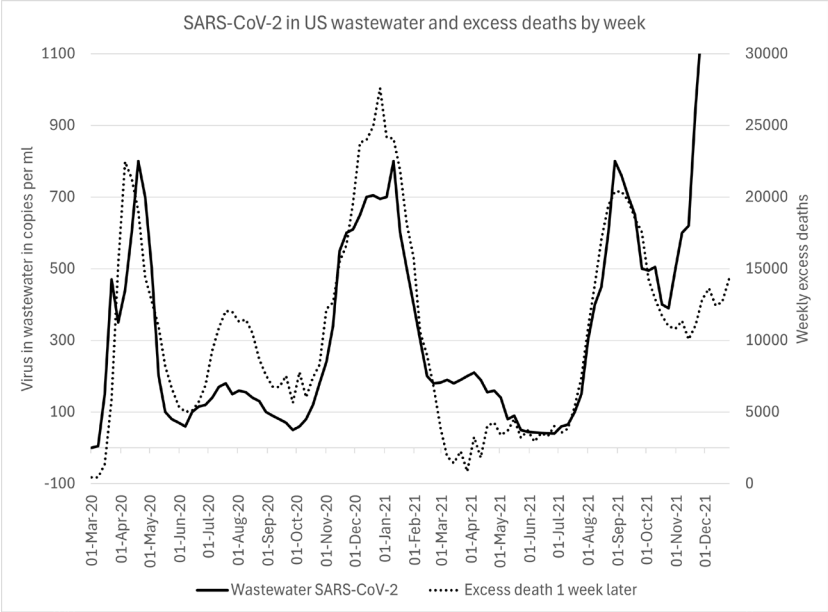

Figure 13: Virus levels in sewage plotted against excess mortality in USA.[1172,1173] *Vaccines rolled out in the first half of 2021.*

Each wave of virus had a comparable wave of excess mortality. There is an outlier period in March and April of 2021. Here there

was virus still present but minimal deaths. That is to be expected when those at risk of dying were injected first and had their infections earlier, contributing to the higher and wider winter peak, while those who are younger continued to be infected but without dying. Summer 2021 saw the arrival of Delta, the statistical illusion washed out and the wave of both virus in sewage and mortality were both comparable to previous waves despite the USA being heavily vaccinated by then, certainly among the groups at risk of dying from covid. The only claim of benefit could come from comparing July and August in 2020 to 2021. The outlier is 2020 when there were more deaths. Too many died then for the amount of virus present suggesting other causes of preventable deaths in 2020. Notably, the reduced hospitalisations in autumn 2021 with monoclonal antibodies did not translate into reduced overall mortality. At the end of 2021, the virus had mutated into the Omicron variant and the virus and death curves properly separated for the first time due to Omicron being less deadly.

COMPARING COUNTRIES

Let us zoom out. If vaccines worked there should be clear differences between countries or regions within them depending on vaccine uptake. Denmark was the first country in the world to permanently stop using the AstraZeneca vaccine on 14 April 2021 and the next day Sir John Bell, a powerful voice in medicine and a prominent member of the UK Vaccine Taskforce, predicted that Denmark,

> *will have a lot of dead people. Many, many, many more dead people than they would have had if they'd used the AstraZeneca vaccine.*[1174]

That did not happen. There are other comparisons that also fail to show the claimed benefit. A neat comparison of deaths in Israel and Sweden showed that throughout 2020 deaths tracked perfectly with twice as many deaths in Sweden as Israel, likely due to

Sweden's older population.[1175] The earlier vaccinations in Israel should have meant that Israeli deaths fell away and stopped tracking the Swedish deaths. However, the ratio stayed totally constant despite the much earlier vaccination campaign in Israel. By the end of March 2021, more than half of Israelis had been vaccinated whereas only 10 percent of Swedes had been, yet the ratio of deaths did not change.

If vaccination had saved lives we could look to see a difference between countries with different vaccination rates. For example, Eastern European countries have markedly different vaccination rates from 30 percent to 66 percent having two doses by the end of 2021, yet it is impossible to predict the covid mortality numbers based on which countries had more vaccinations.[1176] Croatia surrounds two thirds of the Bosnian border. Bosnia, with its more fragile healthcare system, had more overall deaths and more covid deaths in 2020. However, from September 2021, despite Bosnia having half the vaccination rates of Croatia, they had identical numbers of covid deaths.

Other case studies comparing populations with different vaccination rates were also telling. Palestine started vaccinating later and reached around a third of their citizens. It could be compared to Israel to see the impact of vaccination. Not only are the two countries geographically similar but before vaccination the proportion of the population dying in Palestine matched the proportion dying in Israel. Palestinians are on average younger but a more fragile healthcare system meant that deaths were similar before vaccination. The comparison is valid. Despite very different vaccination rates, the covid attributed deaths continued to exactly track in the two countries.

When comparing every country in Europe or every state in the USA, low vaccination regions had the highest mortality in the

Delta wave of summer 2021. This was widely shared as evidence of success. It was not. Plotting the deaths seen in 2020 before vaccines against the vaccine uptake in 2021 showed the same pattern – this meant the pattern was due to other pre-existing differences. It could not have been the vaccine when the effect could be seen before a single vaccine was given.[1177] These differences were because of other risk factors for death whether due to age, ethnicity, underlying health or the healthcare system.

By spring 2022, the relationship had reversed. European countries with the highest vaccination rates had the highest covid-attributed and overall excess mortality.[1178] The low vaccination countries had the fewest deaths. It would have been concerning enough if the wealthier regions had simply matched the mortality levels of areas with higher baseline rates, but in fact, their outcomes were worse. In England, by April 2022, for over 60 year olds, the more boosters a region had, the more covid deaths there were.[1179] The data from summer 2021 which had an illusion of a benefit was widely shared but the later data showing the reversal was not.

Now let us zoom back in and look just at individual US states. California and Florida were frequently compared because California had higher vaccination uptake and lower mortality but no consideration was given to the much older demographic in Florida. Similarly, many news outlets claimed that Republican leaning areas in the USA had far higher covid death rates and claimed this was a national pattern because Republicans were refusing vaccines. However, the research behind these headlines was based on only Ohio and Florida because they are among the few states where voter records can be linked to death records. [1180] Only Ohio showed a strong difference by political group but Ohio is almost all Republican save for a scattering of Democrats in a few large cities. Those cities have younger populations, higher incomes, better hospital systems, higher insurance coverage, more

doctors per capita, and higher vaccination uptake. Furthermore, although the study adjusted for age, the authors admitted that it was limited by not fully accounting for other major factors such as race, income, population density, healthcare access, and existing health problems. [1181] A county that leans Republican might simply be poorer, more rural, and have weaker hospital systems, leading to higher death rates for reasons unrelated to vaccination. The researchers had no access to individual vaccine records to check. Nevertheless, the media turned this limited result into a sweeping national story that penetrated.

If the vaccine prevented deaths then there should be fewer deaths in the year of the vaccine and fewer deaths per covid case. There were 350,000 covid attributed deaths in the USA in 2020 and 470,000 in 2021. [1182] A Gallup poll in 2024 in the USA showed that 25 percent reported having covid in 2020 and 34 percent in 2021 – matching the data on virus in the wastewater. [1183] If people's perceptions were right that would mean that there had been 85,000,000 people who had covid in 2020 and survived to take the survey and 115,600,000 survivors in 2021. Overall that works out as a rate of covid attributed death per case of 0.4 percent in both 2020 and 2021. Where was the vaccine benefit? For 2022, 46 percent said they had had Omicron. As it was a third as deadly that would have meant around 210,000 covid deaths in the absence of vaccine benefit. There were 260,000 recorded covid deaths that year.

As with hospitalisations, it is the mortality from all causes that really counts. Deaths in the first weeks after vaccination were lower for a period in the vaccinated than the unvaccinated because the dying rejected the vaccine. However, in Hungary, after between only 55 and 80 days from vaccination those who had taken Pfizer or Moderna had had more deaths than the unvaccinated.[1184] From 2022, the more vaccines a country had given the higher their

overall mortality.[1185] Data collated from 39 countries shows that the excess mortality in 2022 strongly correlated to the number of vaccine doses given per capita.[1186] The excess mortality was highest in 2021 for nearly half of 47 countries studied from 2020 to 2022.[1187] A quarter saw their highest mortality in 2022, after the vaccines. Doctors may have been less ready to diagnose a death with Omicron after a vaccine as a covid death but if all cause deaths remain high then the vaccine must have not provided a benefit, caused harm or both.

THE ONS DATA MANIPULATION & SURVIVORSHIP BIAS

After considerable campaigning by me and others to see data on deaths by vaccination status some data was released after nearly a year had passed. Prof Norman Fenton, British mathematician, pointed out that there were serious anomalies in the data which he claimed could not be right,

> *somehow the vaccines are not just stopping COVID deaths, but they're stopping non-COVID deaths amongst the vaccinated.*[1188]

Mathematical analysis showed what looked like a combination of a miscategorisation error, where a tiny delay in registering the population as vaccinated could create the illusion of a wave of extra deaths in the unvaccinated.[1189] With the rollout the unvaccinated population in older age groups was shrinking rapidly. If even a small number of expected deaths from natural causes in the recently vaccinated were miscategorised as being in the shrinking unvaccinated population then the unvaccinated mortality rate (deaths per 100,000 people) would rocket temporarily. It would be reasonable when someone is critically ill not to prioritise the accuracy of their vaccine record. Miscategorisation becomes likely in that scenario.

The ONS response to this was to make two contradictory claims. Firstly, they claimed that the unvaccinated mortality spiked because of,

> *a 'healthy vaccinee effect' where people who are ill (either due to COVID-19 or another relevant illness) are likely to delay vaccination.*[1190]

If the dying remained disproportionately in the small unvaccinated population, then of course there would be a mortality spike. They claimed this fully explained the high number of deaths in the unvaccinated that occurred during vaccination.

However, over a longer time period the same ONS data showed there were fewer deaths among the unvaccinated. The same report explained this by stating,

> *Changes in non-COVID-19 mortality by vaccination status are largely driven by…prioritisation of clinically extremely vulnerable and people with underlying health conditions.*

They tried to have it both ways. When the vaccinated died at unfeasibly high levels it was because the sickest were prioritised. However, when the unvaccinated died at disproportionately high levels it was because the sickest were not vaccinated.

The ONS published updates to this data of deaths in the vaccinated and unvaccinated. However, in later updates of the data the proportion of deaths that had been reported for the country as a whole that were included in this data fell off precipitously, especially for the young. Ultimately the data the ONS was using to declare vaccination a success was not representative of the real world.[1191]

The ONS did publish their own analysis. They had access to full vaccination, comorbidity and matched death data, meaning they could have tracked death trends over time to see if mortality actually declined after vaccination. They could have used

straightforward statistical models to test whether vaccination was linked to fewer deaths while accounting for other factors like age and health conditions. They rejected these simple methodologies. Instead, they chose a misleading approach, taking full advantage of the two week danger window and the illusion of benefit seen when the susceptible have their infections earlier.[1192] They compared death rates over just a two-week window, pitting 79-year-olds who were not yet eligible against 80-year-olds who were. However, during their chosen study window, more than three-quarters of 79-year-olds took their vaccines. All of them would have been passing through the two week danger window of immune compromise, the 40 percent increased risk of covid infection that came with that and the short-term increase in deaths that would occur from these earlier infections. By contrast the 80-year-olds had already passed the initial immune compromise phase and were benefiting from the fact the susceptible had already had their infections. Worse, it was not the usual two weeks that were ignored. They failed to state exactly how long after vaccination they claimed the 80 year olds were *"not protected"* for, only vaguely claiming that it varied by age and vaccine type, sometimes taking up to 28 days. It sounded like they adjusted this period to suit.

Why didn't they compare death rates daily? Why limit the analysis to only two weeks? Why overcomplicate the methodology when a simple time-based comparison would have been more transparent? Even with this manipulation, they could only claim a 50 percent reduction in deaths, far below the 80-90 percent figures promoted by the government.[1193] Like many others the ONS was depending on a survivorship bias to make the claims that supported the narrative.

Eventually the UKHSA stopped publishing hospitalisation rates in the unvaccinated and simply compared people by how recently vaccinated they were.[1194] They could show low rates in those

injected in the last 6 months, but the group they had just left i.e. those last infected between 6 and 9 months ago had by far the highest rates. Statistical illusions were being used over and over to make unfounded claims.

WHEN OMICRON HIT SOUTH EAST ASIA AND OCEANIA

Australia, New Zealand and South Korea make a useful comparison group because they were heavily vaccinated before experiencing their first major covid wave in early 2022, during Omicron. By mid-2022, each had recorded around 400 deaths per million despite high vaccine coverage among almost all at risk of dying. Compare that to France and much of Europe, which had their first major wave with the original Wuhan strain in early 2020, before vaccines.[1195] By summer 2020, France and Europe also had mortality rates of under 400 deaths per million. The UK had a worse first wave, with just over 800 deaths per million by the same point. Omicron was estimated to be about one-third as deadly as the original strain, for both vaccinated and unvaccinated people.[1196,1197] So if countries vaccinated ahead of Omicron saw similar death rates to unvaccinated countries hit by the original strain, where was the clear benefit of being vaccinated before exposure?

Hong Kong was frequently cited as proof that vaccination mattered because its mortality in early 2022 was dramatically higher than in neighbouring Southeast Asian jurisdictions.[1198] The claim was that the city's failure to vaccinate its oldest and most vulnerable citizens was to blame. Yet the story was more complex. When Omicron arrived in February 2022, a Zero-covid policy meant nearly every confirmed case, including mild and even asymptomatic, had to be hospitalised or isolated in a government facility.[1199,1200] Inevitably, hospital capacity was quickly overwhelmed. Within days, emergency departments were lined

with patients on trolleys and in outdoor holding areas, scenes that triggered public panic and a cascade of bureaucratic over-reactions.[1201] Reuters reported how,

> *bedridden elderly and children were seen left for hours in carparks, waiting in the cold and rain…*[while] *thousands of people are still flocking to hospitals, worried that they could spread the virus to family.*[1202]

Medical figures urged that infected elderly residents should remain in care homes and be managed by Hospital Authority outreach teams.[1203] Visitors were banned and by the first week of 2021 there were already reports of care home residents losing an average of 3-4 kg in weight.[1204] Médecins Sans Frontières later reported that *"up to 40% of staff"* in some homes were infected during the early 2022 wave, which would have resulted in profound manpower shortages.[1205] The Legislative Council later recorded its,

> *grave concern that some infected residents… were not sent to hospital promptly while some other infected residents were returned… from the hospital without receiving any treatment.*[1206]

As hospitals were overwhelmed by the worried well, it is likely that admission became more difficult not only for those testing positive but also for residents with other acute conditions, such as dehydration, heart failure, or bacterial infection. The inevitable result was a wave of preventable deaths. By the end of the surge, about 10% of all residential-care residents in Hong Kong had died, one of the highest long-term-care mortality rates recorded anywhere.[1207] This outcome was the predictable result of a zero-covid hospitalisation policy that filled wards with the well, abandoned the truly sick, and collapsed the care system for the elderly.

Like elsewhere, Australia's death wave had more to do with policy than with the virus itself. In aged care there were severe workforce

shortages, with surveys showing around a quarter of staff absent, ranging from between 15 and 50 per cent in different facilities.[1208] Hospital staffing was also hit. The Australian Nursing & Midwifery Federation noted that,

> *with staff being furloughed due to infection with COVID-19 or designation as a close contact, there were no other staff available to fill shifts or cover them… staff furloughs and isolation requirements have created enormous staffing pressures across the health system.*[1209]

With too few staff to run wards safely, hospitals became unable to admit patients, leading to widespread ambulance backlogs. A Victorian review of emergency services showed this was not only due to hospital 'ambulance ramping' but also to pandemic protocols, saying,

> *COVID-19 caused extra work for paramedics to thoroughly clean their vehicles and equipment, and re-stock… This delayed response times for some ambulances, on top of increased ambulance ramping at hospital.*[1210]

The same report highlighted the impact of call-handler furloughs,

> *On 14 January 2022 [the service] received 2501 calls, with 139 calls waiting more than 10 minutes… the longest waiting over 76 minutes.*

It also acknowledged that fear drove demand, noting,

> *Little wonder, perhaps, that the first response of many facing a loved one displaying symptoms was to see this as an emergency and to ring 000… sometimes this was warranted, at other times, not.*

The outcome of all these factors in terms of mortality was predictable. The review in Victoria identified 33 patients whose deaths were attributed to delays in care but acknowledged that further deaths linked to the initial delays would have occurred after hospital admission. In short, deaths rose not because Omicron was uniquely lethal, but because the system disabled itself and could not deliver timely care.

As we have already seen, the immune changes from repeated vaccination increased the risk of infection in the vaccinated. Low vaccination rate countries stopped reporting on covid in 2022. In the meantime, in high vaccination countries the first Omicron wave had a similar mortality to previous waves as deaths from policies continued to create an illusion of a deadly virus. Only as healthcare systems slowly returned to previous practices did the mortality start to return to baseline.

Deaths that were the result of under or over treatment could not be prevented by a vaccine. Although the medical literature was indeed littered with studies showing vaccination worked to reduce cases, hospitalisations and deaths, other evidence shows this was not the case. Taking a step back and looking at the impact from every other direction tells a very different story. However, there seemed to be an inability to look up from these studies focused on a particular group to sense check their findings against what was happening in the real world.

Questions arising:
Were the early expert warnings correct that a genuinely effective vaccine was never achievable on this timeline?
Who will be held to account for the vast public funds poured into a product that failed in its primary purpose?
If lives were not saved, how can the harms be justified in the name of the "greater good"?

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HOW WERE SCIENTISTS CONVINCED THE VACCINES WORKED?

We have not lost faith, but we have transferred it from God to the medical profession.
George Bernard Shaw, 1906[1211]

The illusion of benefit started as prophecy based on modelling and a statistical illusion but it was maintained through bias in the way data was handled in scientific papers. Healthcare workers frequently relied more on peers, institutional sources and media than on primary scientific literature, with only a small subset of doctors citing medical journals as a key source of information.[1212] Like others, even doctors are not immune to prioritising

information that reinforces their prior beliefs, keeps their careers secure and is acceptable within society.[1213]

The High Priests could cite numerous papers supporting the covid vaccines in the medical literature, giving an illusion of credibility. There are too many papers to take them apart individually. However, the issues recur. Scientists were convinced because papers with a range of serious and undermining biases were laundered through the peer review process into the medical literature. Here we will look at the problems with peer review then step through each of the key categories where there was serious bias.

THE ILLUSION OF PEER REVIEW

Many assume peer review filters out all papers relying on bias, but it does not. Worse, as well as acting as a gate keeper, peer review could also put the brakes on papers that countered the narrative. Pro-narrative research was fast tracked in days while, on the rare occasion where a counter narrative paper was published the process would take 18 months or more.[1214,1215] Peer review cannot sanctify research papers. All it aims to do is catch mistakes privately to avoid embarrassment. Peer reviewers often do not even see the raw data let alone repeat the experiment, so their rubber stamp is not an endorsement of the results. Furthermore, peer review did not stop discussion sections contradicting the papers findings. These sections would be quoted as fact in media even thought they were closer to opinion.

The credibility of peer-reviewed articles has been questioned for years even by those responsible for maintaining the standards of the review process. In 2015, editor of *The Lancet* Richard Horton suggested half of the scientific literature might be untrue.[1216] The *New England Journal of Medicine* faced criticism from former

senior editors for downplaying conflicts of interest. Dr. Marcia Angell, who served as editor-in-chief for two decades, argued in 2009 that,

> *It is no longer possible to believe much of the clinical research that is published, or to rely on the judgment of trusted physicians or authoritative medical guidelines.*[1217]

Nature reported that over half of researchers could not provide raw data supporting their work.[1218] If peer review was intended to help us share the truth it was failing long before 2020.

Peer review even caused changes of results without explanation. For example, an Israeli study in 2021 showed that the previously infected had no extra benefit from vaccines. The preprint publication clearly showed that those who were infected after a vaccine had weaker protection from subsequent infection – a form of Original Antigenic Sin.[1219] The data in the preprint paper was presented in a graph that showed reinfection rates over a longer time frame for the unvaccinated than the vaccinated which obscured this important finding. That finding disappeared altogether in the peer reviewed paper.[1220]

Peer review acts as a gatekeeper. Certain data was denied simply because of who said it or how it was said. There was great difficulty in having work included, even on preprint servers, let alone in peer reviewed journals. Often the authors carrying out the analysis had expertise from a broad range of non-medical fields and despite the quality of their work would be dismissed on the basis of their lack of medical qualification. Analysis based on official data sources would be rejected as not being 'research' by certain publications because there was no new data in the paper. The same journals would consider publication of such analysis as an opinion piece but then would not permit presentation of graphs. A huge amount of valuable analysis was therefore carried out and published independently outside of the peer reviewed literature. Much of the

time this work was only ever read by a minority who were already curious.

Imagine two extremes: people who judge food only on taste and those who only judge on appearance. The same is true for the consumption of information. A minority enjoyed all the best tasting data analysis even when the source did not have credentials, had not been published by a big name journal or whose data was not polished with graphic design skills. They dug into raw evidence, savouring insights peer-reviewed work often missed. The majority would dismiss any such information without even tasting it, trusting only polished, peer-reviewed papers. Their dismissal was helped along enormously by big tech companies labelling such information as poisonous. The minority, could also tell you that much of what was presented beautifully in peer reviewed papers looked impressive but did not taste good.

There was an enormous body of work which should never have passed peer review that was published in weighty journals. The majority of scientists and doctors trusted the sources, read the abstracts and believed them without ever reading on critically to discover the flaws. For example, the media tried to suppress evidence of a US grant, DEFUSE, which funded research in Wuhan. In 2024, a survey was published of 168 experts in virology, infectious diseases or genetics who were asked what evidence they were familiar with.[1221] While a third claimed to be familiar with a fake control paper only 22 percent said they knew of the DEFUSE proposal. Experts had failed to read beyond the PR published by the media. That created an illusion of consensus but it had nothing to do with actual science.

A form of peer review took place in open forums using social media and has allowed the public to see the attempts at hoodwinking that certain scientists were brazenly using in their publications. It is

high time that scientists embraced a more transparent form of peer review and critique allowing for serious challenge.

WELCOME TO THE HOUSE OF BIAS

Now let us turn to the papers themselves. Superficially credible vaccine studies are riddled with bias, statistical sleight of hand and an unwillingness to subject convenient conclusions to the same level of rigorous analysis as inconvenient ones. Statistics were twisted, inconvenient numbers were buried and critics were silenced. Peer review has too often been used as a shield for unchecked assumptions and flawed methodologies. The illusion of overwhelming vaccine success came about because scientists were blind to the biases in their own work.

I cannot guide you through each and every paper, so instead I've provided a checklist of the tricks used, along with key examples. The same flawed methodologies appear in almost all scientific papers and official public health reports claiming covid vaccine success. Once the nature of these distortions is understood, many of those claims can be readily dismissed.

The illusion of benefit was constructed through a variety of statistical distortions which I hope to illustrate with an analogy. There are seven major biases which enabled claims of vaccine efficacy as well as prejudice in how the data was interpreted:

1. The two week danger window
2. Biases in how adjustments to data were made
3. The denominator bias
4. The numerator bias
5. The healthy vaccinee bias
6. The time lag bias
7. The invented data bias

To explain these I am going to show you around the House of Bias. As you approach the large house you notice there are people waiting outside who do not look keen to go in. The downstairs is full to bursting with people of all ages and states of health and a lady at a desk registers people as being present. Nurses come down the stairs inviting people to come up for their vaccines. You see incredibly frail people dragging themselves up those stairs, with help or on their own but there are others who are on death's door who decide to stay behind. When your turn comes you are taken upstairs, injected in the corridor and told to wait. A full two weeks later you are finally allowed into the room labelled 'first dose'. In due course, a nurse invites you to climb another flight of stairs for another corridor injection. Again two weeks pass before you are invited into the room labelled 'second dose'. The routine repeats with each dose.

1. THE TWO WEEK WAIT BIAS

The first bias we have discussed before and it occurs with any covid labelled illness and death that accumulated while people waited in the upstairs corridor. After the first dose, that two week period was critical as it was the danger window when people were at much higher risk of infection. Most of the time these additional infections were attributed to the 'unvaccinated population'. Likewise, any covid within two weeks of the second dose was attributed to the one dose group and so on. Because of the higher rates in the first two weeks this methodology not only underrepresents illness and death in the vaccinated groups but also artificially inflates the risk among the unvaccinated, creating a double whammy effect, creating the illusion of benefit. Even a placebo would be effective with this type of measurement.[1222] In fact, even a drug that made infections worse could look beneficial with this methodology.

2. THE ADJUSTMENT BIAS

The second bias was ignoring the fact the unvaccinated were not a homogenous group. As a whole, the unvaccinated differed from the vaccinated in several ways. Looking around at the people left downstairs, they were younger on average, more likely to be from ethnic minorities, more often urban and disproportionately poorer. However, this hides major differences within the category. The unvaccinated represented extremes – opposite ends of various bell curves. There were rebels who instinctively resisted government directives, alongside the highly religious, who were concerned about fetal research and committed to following their faith's rules. Some were intellectuals who leaned on their understanding of molecular biology to reject the vaccine, while others made their decision based solely on intuition and gut instinct. The group included athletes, highly cautious about introducing chemicals into their bodies, as well as individuals in such poor health that they saw little reason to intervene, knowing death was imminent. As the latter group passed away, their proportion declined rapidly, making any direct comparison with the vaccinated unfair. Many studies failed to account for these biases properly.

The most trustworthy scientific papers ensure their study population accurately represents the population of interest and present the raw data showing differences between groups. If the sample is in some way unrepresentative, with, say, overrepresentation of higher socioeconomic groups, minor adjustments i.e. inflating the covid death rate can be made to correct for discrepancies. These are reasonable for minor sampling issues.

However, if the sample differs significantly from the overall population, necessitating extensive adjustments, the study effectively ceases to be based on real-world evidence. Instead, it

becomes another prediction based on modelled data. For example, if a sample of young people is used to estimate covid mortality in old people, the mortality rate could be multiplied many times to reach a reasonable number. The assumptions on how much each amount should be inflated by can dominate the numbers. The numbers used in these adjustments were only rarely published. One extreme example was when the CDC claimed, contrary to all other evidence, that people with a prior infection were over 5 times more likely to become infected than those who had been double vaccinated.[1223] The actual results from the study showed a 70 percent higher rate but their adjustments made this into 400 percent. (There were other serious flaws in the study which makes even the 70 percent claim highly dubious).

In study after study of vaccine effectiveness the covid outcomes for the unvaccinated were inflated to account for the fact they were younger and healthier. Adjustments like this have even been used to reverse the findings of studies and to make extreme claims for vaccination effectiveness. Meanwhile, adjustments to account for the higher risk in the unvaccinated because of socioeconomic factors or ethnic status and the fact a proportion rejected it because they were close to death were never carried out.

At a population level, the vaccinated died substantially more because they were older. A fair analysis required adjustments. Adjusting for age alone made it look like the unvaccinated died of covid more. However, they died of non-covid deaths more too. Adjusting for age and frailty was key. Importantly, being more frail increased the risk from covid death more than the risk from non-covid death. Adjusting for the true impact of frailty on covid deaths showed no benefit.[1224]

3. SHRINKING THE DENOMINATOR

The real world claims that the vaccines were effective were based on the proportion of the vaccinated population with covid compared to the unvaccinated population – the so called 'case rate'. It is a fraction – the cases divided by the population. Changing either the numerator, the cases, or the denominator, the population, can be used to cheat the result. Miscategorising the two week period changed the numerator but the devil's in the denominator.

Underestimating the size of the unvaccinated population boosts the unvaccinated case rate. There is a population who sit outside the government's data collection systems. However, they were not ignored completely. If they caught covid, had a vaccine or died they would enter the government systems. The number of "*unlinked*" covid cases in the UK data was always significant as people who had not been in the system appeared in the data for the first time. Data that would allow a reasonable estimate of the real population size, like the food consumed or sewage levels produced, used to be published but was no longer disclosed.

No one knows how many unvaccinated people there are. The closest we can come is by estimating the whole population size and subtracting the number who had a vaccine. But there was a problem. It is very easy to underestimate the size of the population in a country. In fact, so many people were first registered in the system when they were vaccinated that for certain age groups in multiple regions there were more people who were vaccinated than the ONS claimed existed![1225] The 2021 census was a chance to correct for that obvious error but the ONS continued to claim far fewer people existed than had been vaccinated for certain age groups.[1226] This meant that when calculating the percentage of unvaccinated who were admitted to intensive care, the fantasy

denominator was so close to zero that the rates became ludicrously high.[1227] It was a very dishonest way to present the data. Public Health Scotland even estimated the unvaccinated population as a third larger when calculating death rates than when calculating case rates in the same week.[1228] With a choice of denominators the result will be influenced by the bias of the person choosing.

The difficulty with estimating the population is a global one. The place with the least accurate population measure is likely to be New York City. The total population size was massively underestimated. Therefore, the claimed size of the unvaccinated population, calculated by subtracting the count of the vaccinated, was far too small. The city used 2019 population estimates in their calculations of case and hospitalisation rates. Had they used their own 2020 estimates the size of the unvaccinated population would have increased by half a million.[1229] The result is that calculations of cases, hospitalisations or deaths as a proportion of the population ended up massively inflated. Proof of this comes from the fact that the hospitalisation rate in January 2022, with Omicron, was claimed to be five times higher in the unvaccinated than it was during the spring 2020 peak when everyone was unvaccinated.[1230] The claims made about the death rates in the unvaccinated in New York City were that the deaths were 30 times higher than the highest ever covid death rate in the whole of the USA. It is a ludicrous claim.

Massively underestimating the size of the unvaccinated population could explain why New York City were still making bold claims about vaccination working against Omicron while Iceland, where the population is easier to count, were showing *higher* case rates in the vaccinated population.[1231, 1232]

The BBC commissioned a survey which used a weighted population to be representative of the British public. This survey

showed that 20 percent of adults were unvaccinated in July 2022.[1233] This was exactly the same estimate that the UK public health authority the UKHSA vaccine database had.[1234] However, the statistics institute, the ONS estimated only 8 percent were unvaccinated.

How could this affect the results? Let us look at the case of a placebo (see table 7). Imagine a population of 10,000 people. 8,000 are vaccinated with a placebo and 20 percent, or 2,000, are not. There are 80 cases in the vaccinated group and 20 in the unvaccinated – 1 percent of each group. The cases remain constant because people were measured whereas the denominator was only ever an estimate not a count. However, not all 10,000 people are in the estimate. All 8,000 vaccinated are but only 696 or 8 percent of the total are included as unvaccinated in the calculation. The same 20 cases in the unvaccinated are all included but represent nearly 3 percent of 696. The 80 cases in the vaccinated represent only 1 percent because all of their denominator was included. Underestimating the unvaccinated population by this amount risks increasing the case rates of the unvaccinated by nearly three times.

This one faulty assumption would give a placebo vaccine, with zero efficacy, an apparent efficacy of 65 percent i.e. it would look like two thirds of cases were prevented by the vaccine. The same would apply for calculations of hospitalisations and deaths. The percentages would differ by age group, but the principle is the same.

	Actual hypothetical population	**ONS assumed population**
Whole population	10,000	8,696
Vaccinated	8,000	8,000
Unvaccinated	2,000	696
Cases vaccinated	80	80
Cases unvaccinated	20	20
Cases as % vaccinated	1%	1%
Cases as % unvaccinated	1%	3%

Table 7: Demonstration of how an underestimate of the size of the population can distort the case rates

There was another way to distort the denominator. Jonathan Engler and Joel Smalley, both of whom I know well, had two doses of vaccine, which they had never taken, erroneously entered into their medical record by 2022.[1235] No one was interested when they tried to draw attention to the unauthorized access of their medical records, incorrect data entry and the diversion of NHS funds. By 2024, Jonathan had had two more phantom doses added to his record.[1236] Each of the four doses was apparently administered at a totally different site – places he had never visited. It is impossible to know how extensive these phantom data entries were. Every incorrect record would increase the denominator for the vaccinated population, lowering their case rate. At the same time it would decrease the denominator for the unvaccinated population, increasing their case rate. Despite Engler's efforts to draw attention to this, as yet, a much needed investigation into this problem has not begun.

4. BOOSTING THE UNVACCINATED NUMERATOR

The numerator could also be manipulated with covid cases and deaths exaggerated among the unvaccinated. People who had vaccines did not all have this properly recorded in their medical record. The government had to release guidance on what to do when patients arrived months later for their second dose and were not in the system.[1237] That meant covid and other deaths in the vaccinated were being added to the tally for the much smaller unvaccinated group.

The UKHSA data on hospitalisations and deaths by vaccination status appeared to show marked vaccine benefit. However, as well as problems with the denominators there was a numerator issue. People who tested positive but were not in the system were recorded as *"unlinked"* cases because they did not have a medical record. However, once they were hospitalised they became linked to one. Therefore, the hospitalisations and deaths included all the unlinked people in the numerator as unvaccinated but the population measure did not include the unlinked in the denominator. Furthermore, whenever there was a problem matching a vaccine record to a hospital record that hospitalisation or death was recorded as unvaccinated too.

5. HEALTHY VACCINEE BIAS

Imagine how many people die within 28 days of a trip to the hairdressers. Not many: people who are about to die tend not to be fit enough to go and get a haircut. Therefore, in the period within days of a haircut the mortality rate will be very low. After a few weeks it will be higher but only after months will it reach the expected rate. Even people who have managed a haircut in the last year will be disproportionately less likely to die. With every vaccine dose there is a period of weeks or longer where there are fewer deaths than a normal population would have. Remember back to

the house of bias where the vaccinated were shuffled from corridor to rooms. Imagine the room where the first dose was given contained 80 year olds, some quite chipper, some frail but some already close to dying. The nurse comes in offering the elusive promise of a healthier future by taking them to the second dose room. A while later the first dose room would have three types of people. First, those close to death who did not have long enough prospects to benefit. Second, those who were harmed by the first dose. Finally, there might be a small group who figured out that the claims were oversold after having already had one dose. The result is that the dose 1 room has far fewer people in it but a very high proportion of the deaths. In the meantime, the dose 2 room has plenty of people and very few deaths for a period.

Therefore, the higher risk of dying from any cause needs to be factored in before declaring prevention of covid deaths or else claims could be made based on bias alone.[1238] A highly influential study from Israel claimed a 90 percent lower mortality in those who had boosters.[1239] However, a reanalysis showed that there was a similar massive reduction in mortality due to non-covid causes.[1240] That implies the difference was bias between the groups e.g. the dying rejecting vaccines and thereby giving the non-boosted population a very high mortality rate. The original authors had access to this data but failed to mention the bias at all.

There is also a permanent healthy vaccinee bias. The vaccinated represent a healthier subgroup who were less likely to be ill or die compared to the unvaccinated before they were injected. This bias remains but is rarely accounted for.

The idea of a healthy vaccinee effect was utterly distorted. It should mean that the vaccinated have fewer diagnoses, because only the unhealthy defer their injections. However, Pfizer turned

it on its head claiming it meant *more* trips to the doctor and therefore could explain *higher* rates of heart arrhythmias.
They wrote,

> *healthy individuals interested in vaccination would be more likely to consult and therefore have arrhythmia diagnosed.*[1241]

Again we have an unfalsifiable situation – whatever the results showed could be ignored. When there are fewer diagnoses they are ignored because the vaccinated were healthier so they could conclude the vaccines were safe. Evidence of more arrhythmias is ignored because the vaccinated attend the doctors more often. There was an excuse for every data signal. In no scenario could the vaccines be a problem.

6. THE TIME LAG BIAS

Vaccination is postponed for people who have already tested positive for covid or have symptoms. They get left behind in the previous room. There is therefore a considerable time lag built into any real world data on effectiveness. Of course the covid deaths in the weeks afterwards will be higher in the unvaccinated cohort because people who caught covid just before becoming eligible did not get vaccinated. This important bias was not only ignored but used to present data that claimed the vaccine was working![1242] Even if the vaccine had been a placebo this bias would give another illusion of benefit. The high rates of covid hospitalisations in the first two weeks after injection are all the more striking once this bias is taken into account.

A common tactic to make vaccines appear more effective in a study was to begin the study during peak cases when everyone was unvaccinated. As time went on infections were falling. People would progressively move into the vaccinated group and cases would be measured during a period when there was a lower possibility of infection. At an extreme summer covid in the

vaccinated would be compared to winter covid in the unvaccinated. The same trick was used in 1918 as we saw in *Chapter 25*. Again, a placebo would look effective with that methodology.

The ONS used this method to claim a 32-fold lower mortality rate among the vaccinated, in a report in November 2021.[1243] The Office for Statistics Regulation upheld a complaint about this manipulation but the report was never amended.[1244] As if this massive exaggeration was not bad enough it was inflated by a journalist on prime time television saying the unvaccinated were 32,000 times more likely to die.[1245] That would mean one death in the vaccinated would equate to 32,000 deaths of unvaccinated people. The month before, 86 percent of covid deaths in over 60 year olds had been among the vaccinated.[1246] This was utterly incompatible with the ONS claim but this contradiction was never addressed.

7. THE INVENTED DATA BIAS

Where actual science was not producing the desired results there was always the opportunity to fall back on modelling the results based on fantasy assumptions. Numerous studies simply took the assumption that the vaccines prevented 95 percent of infections and then extrapolated from there to produce fictional graphs of how wonderfully successful they were.

Sometimes the modelling was more extreme, like the WHO modelling which inflated deaths in Africa to give higher overall excess - the estimated excess was up to twenty fold higher than recorded covid deaths.[1247] This same method, of assuming that non-western countries had a higher mortality based on no evidence, had, again, been used to inflate deaths from the 1918 influenza as we saw in *Chapter 25*.

Other data was invented to feed a politicised agenda. Rochelle Walensky, while CDC Director, said in 2021,

> *this is becoming a pandemic of the unvaccinated*

while her colleague, Jeff Zients continued,

> *and we're seeing it in the data. Unvaccinated Americans account for virtually all recent covid-19 hospitalisations and deaths.*[1248]

That lie was fed by the fact that the US, like the UK, included every patient with no matched vaccine record in the hospital system as an unvaccinated patient. In June 2023, Rochelle Walensky finally admitted,

> *We still, to this day, do not have data on people who are coming into the hospitals who are vaccinated. That is a data point we have lacked.*[1249]

It was never a pandemic of the unvaccinated.

OTHER BIASES

Working through this checklist of bias is enough to discredit the vast majority of the huge volume of covid vaccine research. Where a paper is seemingly innocent of all the above manipulations, it is worth checking if it was written by the pharmaceutical company. Industry backed papers often seemed to produce different results to independent researchers. Many such papers were promoted through press releases without critique such that sceptics could not criticise methods or results as they were not in the public domain while the press release was being shared. Furthermore, independent commentators have questioned whether some studies might have relied on synthetic patient data.[1250] Such datasets begin with real records but deliberately scramble them, leaving what is effectively model-generated information. Yet several of these studies have declined to share their underlying data on the grounds of patient privacy – a rationale that would be difficult to sustain if the data really were synthetic. Regardless of whether those

concerns prove correct, the lack of transparent, auditable datasets is systemic and provides yet another way in which bias can be concealed.

THE VACCINE WORSHIP BIAS

Aside from data biases, there was a bias in how results were presented. When someone becomes ill or dies the doctor making a diagnosis or writing the death certificate takes every piece of evidence into account. There was huge faith in the potential of these vaccines and there was therefore potential for a bias in how a death diagnosis is made depending which room of the house of bias the person was in.

The vaccine worship bias extended into publications in the medical literature. For instance, a study in the *New England Journal of Medicine* showed unvaccinated care home residents had similar or lower covid rates compared to vaccinated ones. Yet the authors illogically concluded the vaccines had protected the unvaccinated even more than the vaccinated(!).[1251] More vaccinations were needed regardless of the results. Such reasoning made it impossible to falsify the hypothesis. Science was dead and vaccine worship had become a religion.

THE OUTCOME

The result of ignoring the biases was a preordained conclusion, propped up by manipulated data and selective reporting. Scientists and doctors who are trained in scepticism too readily accepted findings they wanted to hear without critique. Science should not be about faith – it is about evidence and scrutiny. When you look at the actual measurements rather than the models, the story unravels. In January 2025, Chris Whitty referred to the idea of vaccines reducing infections and therefore transmission as

"blindingly obvious".[1252] When scientists decide the answers are obvious they can stop looking rationally and critically at the evidence. Lazily reporting superficially encouraging results could be the result of being blinded by faith but it becomes impossible to distinguish from intent to blind others to the truth.

If we want genuine science there must be transparency, openness and civilised debate. Without them, we will continue to be sold the same illusions, blinded by dark data and forever lost in a house of bias.

Questions arising:
How dispassionate are any scientific papers based on observational studies?
What if some of the effort and money spent on observational studies had instead been spent on independent randomised clinical trials?
Why did pharmaceutical sponsored papers report results which contradicted independent papers?

PART SEVEN:

THE END THAT NEVER CAME

Part seven follows the quiet retreat – how programmes were wound down because people stopped complying, how the machinery for the next *"emergency"* was kept running, and how politicians avoided debate while insisting nothing had gone wrong. We begin with the forces that drove the silent rollback, then turn to the plans for the *"next pandemic"* and end in democratic debates, where faith replaced scrutiny, leaving us primed for the same catastrophic response to unfold again.

31

THE SILENT ROLLBACK

Nothing is so permanent as a temporary government program.
Milton Friedman, 1984[1253]

From 2022, the already low threat from covid was reduced to a negligible level with the much less deadly Omicron variant, yet, despite no fourth dose recommendation for healthy under 75 year olds, there was a recommendation to inject every primary school child by early 2022.[1254] Worse they continued working down the age groups to babies. The juggernaut seemed unstoppable but people stopped rolling up their sleeves. By November 2022, less than half of Scottish frontline healthcare workers had had a dose that year.[1255] Equivalent data for England was not shared. Let us take a look at how the rollback progressed over time, what drove it and how to return to a more rational approach.

Throughout the vaccine rollout HART was heavily critical of both the efficacy and safety claims and wrote many open letters trying to share evidence of the issues. In September 2022, one of our articles seemed to provoke a change in direction. The article set out a calculation based on official data sources of how many people, by age group, would need to be vaccinated to prevent a covid death – called the *'number needed to vaccinate'*. From an individual's viewpoint this is the easiest way to understand the benefit. Based on all the evidence that number would be huge – close to infinity, because, despite millions, even billions of vaccines, the benefit is negligible. Disregarding the significant evidence that the official data was untrustworthy, as outlined earlier, HART used the same official data to make a point. I calculated a number needed to be vaccinated based on the UKHSA *claimed* covid death rates and deaths per 'case' in different age groups in September 2022 (see table 8). [1256]

The result was the number who would need to be vaccinated to prevent a single Delta death was 230 for over 80 year olds, thousands of 50-69 year olds, tens of thousands of under 50 year olds and nearly 100,000 under 30 year olds. I must emphasise that the real answer was infinity but these figures were the best case scenario presented by government. For Omicron it was still only 275 for over 80 year olds but was between two and four times as high as it had been for the Delta calculation for other age groups. The negative result for children means the vaccinated were dying more that the unvaccinated. I have not been able to unpick how much of that effect is because children at risk of dying were all vaccinated.

Age Group	**HART estimate of number vaccinated to prevent a Delta Death**		**HART estimate of number vaccinated to prevent an Omicron Death**		**UKHSA modelled Omicron estimate**
	deaths per 100,000 people in each group	deaths per covid positive 'case'	deaths per 100,000 people in each group	deaths per covid positive 'case'	number to prevent 1 severe Omicron hospitalisation
<18	negative	negative	negative	negative	>100,000
18-29	93,000	113,9635	310,788	562,771	166,200
30-39	27,000	116,534	132,371	330,845	87,600
40-49	10,000	88,685	42,749	102,172	53,700
50-59	2,600	7,218	8,279	37,466	18,700
60-69	1,300	9,155	2,579	19,417	5,700
70-79	513	520	800	6,682	2,500
80+	230	286	275	2,388	

Table 8: HART and UKHSA estimates for the number needed to vaccinate with two doses to prevent a single death or severe hospitalisation.

A month later UKHSA presented similar calculations to the JCVI which were eventually published in January 2023.[1257] Instead of

deaths, they estimated the number who had to be vaccinated to prevent a single hospitalisation or severe hospitalisation which they defined as needing oxygen, ventilation or intensive care. They did not use the data to reach their estimate but modelled it based on assumptions. Nevertheless their results closely matched HART's death figures. These calculations should have driven rational decision making but they were not done earlier.[1258]

It was too late for the JCVI to reverse their autumn 2022 vaccine rollout plan which had begun a month earlier in September 2022. The Autumn campaign recommended primary doses to healthy children over 5 years of age who had been eligible for a primary course since February 2022. Booster doses were also offered for healthy children over 5 years old who were living[1259] with *"people with immunosuppression"* and health and social care staff and carers over 16 year olds who had already had up to four doses. Just two months later, on 8 November 2022, the JCVI advised stopping the third dose offer for healthy under 50 year olds citing lack of uptake as a reason but adding they would be *"of limited ongoing value"*.[1260] By the end of December 2022, booster doses were stopped for healthy adults under 50, but earlier that month primary doses were offered for the first time to children from 6 months to five years of age if they were thought to be at increased clinical risk.[1261] Even a child with serious medical problems would be hard pressed to be at a similar risk of being diagnosed with a covid-labelled death as a healthy 40-49 year old who was no longer eligible.[1262]

The messaging remained devoid of evidence based reasoning and was painfully dumbed down throughout. Why should the public trust officials who think that *"Get Boosted. Get Boosted. Get Boosted"* is an adequate way to communicate about anything? The constant repetition of *"safe and effective"* in a situation where there was obvious uncertainty was another blow to public trust and confidence. A loss of trust could be measured by how many were

prepared to roll up their sleeves. In spring and autumn 2022, the government continued to try and persuade people to take a first dose saying *"it's never too late"*.[1263] Uptake remained high in the older age groups but of those eligible for a fourth dose under the age of 40, only about 40 percent took one in the UK.[1264]

At the outset the UK government said 15 million jabs were needed to cover those at most risk and in autumn 2022 only 15 million jabs were indeed given.[1265] There was an interesting period where people were declining the injections without being able to fully articulate why. Amusingly, this also applied to certain characters who were public evangelists for the vaccines who were unable to properly articulate why they had stopped taking any more.

The rollback continued into 2023. On 12 February, the offer of the first two doses was stopped for all healthy under 75 year olds including children.[1266] Between September 2023 and January 2024 only 31 percent of NHS frontline staff took a covid vaccine.[1267] By the end of November 2023, 38 percent of individuals aged 65 and over had rejected the latest booster.[1268] For the public who drove the rollback, there came a point where continuing to believe that infections were prevented by vaccination could only be described as delusion and denial.

In December 2023, the JCVI finally did their first cost benefit assessment.[1269] The cost of care from vaccine harm was not part of the equation. The £25 cost of the vaccine was simply pitted against the hoped for savings from reduced covid care. Even based on the exaggerated claims there was not enough benefit to justify spending £25 on a dose for anyone healthy under the age of… wait for it… 90 years old. Nor was there enough benefit for under 80 year olds in a clinical risk group, nor anyone under the age of 65 with immunosuppression. Nevertheless, from 2024, boosters were

still offered to adults over 75 years of age, care home residents and those with severe immune problems.

By the end of November 2024, 86 percent of healthcare workers had rejected the autumn offer.[1270] The wind-down was officially based on healthcare economics rather than MHRA withdrawal. Because the argument was a financial one, booster doses could still be accessed in the private sector for anyone over the age of 12 into 2025.[1271]

The USA story is very different partly because of the privatised healthcare system which meant the economic reasoning could not be used as an excuse to wind-down. In September 2022, the USA had already been vaccinating healthy children between 6 months and 5 years old for three months since June and had been recommending boosters to over 5 year olds since May.[1272,1273] From December 2022, the USA even authorised boosters for anyone over 6 months of age and this offer continues in 2025.[1274] The CDC's vaccine advisors voted unanimously to add covid vaccines to the US childhood immunisations schedule in 2022, with first doses at 6 months of age. [1275] In some states compliance with this schedule is expected for school attendance. Access to University was also predicated on covid vaccinations in many parts of the USA into 2025. The regulators failed to withdraw the drugs but the choice that should come with a private system was overridden with mandates.

A mandate for US healthcare workers expired in June 2023. Gone was the threat of no jab no job that had led to hospitals losing their laboratory services and maternity ward closures.[1276] Afterwards, during autumn and winter of 2023-2024 a CDC survey showed that only 15 percent of healthcare workers and 10 percent of nursing home staff received a booster.[1277] However, 9.5 million US children had the latest booster leading up to April 2025.[1278]

The UK's vaccine rollout overreached, but the USA took it further, embedding covid shots into childhood schedules and education requirements despite weak justification. Neither country based the drive to vaccinate on solid evidence – ignoring the massive numbers needed to vaccinate to prevent a single death, sidelining risks from the vaccines themselves, including unknowns and skipping proper risk-benefit calculations.

Throughout the rollback it was as if the public stepped back and then the advice followed rather than the other way around. The eventual rollback in the UK, with boosters limited to over-75s by 2024, only exposed what was flawed from the start: had a cost-benefit analysis been done earlier, the retreat might have come sooner, sparing millions unnecessary jabs. Had it been done at the start, the whole programme would have been vastly different.

IN RETROSPECT

In July 2025, Stanford professor John Ioannidis teamed up with public health experts in California and Italy to produce an estimate of lives saved from vaccination. He had first published on the exaggerated risk from the virus in July 2020 – although he assumed every covid labelled death was entirely due to covid.[1279] From that time he and others had the data to clearly show the lack of potential benefit. Not only did he wait five years to make this point but his team made yet another model, this time assuming the vaccine prevented 75 percent of deaths pre-Omicron and 50 percent post-Omicron. They concluded 2.5 million lives had been saved globally. These authors set down the tiny numbers of lives that could have been saved in younger age groups if their assumptions of effectiveness were correct. The fact that young people had a low risk of dying in the official data meant that even an effective vaccine would have a low chance of saving a life. They

concluded, based on the assumption of an effective vaccine, that for under 30 year olds,

> *1 death* [was] *averted per approximately 100 000 vaccine doses.*

Finally headlines acknowledged some of the issues,

> *'Aggressive mandate' of Covid vaccines was 'probably a bad idea' as new study reveals WHO mandated blanket vaccination saved 'fewer lives than expected'*[1280]

Ioannidis and his team could have published a similar analysis four years earlier but did not.

Going forward lessons need to be learned about the dangers of a pedal-to-the-metal project. No government project should be begun without a risk benefit analysis but there needs to be more. Serious thought needs to be put into the consequences of starting a project – what direction might it take us in? What if it runs far beyond the initial intentions? What future projects could it justify? No project should begin without clear boundaries and strong brakes.

The public did not reject the evidence of their eyes and ears and as time went on were not so easily taken in. The public saw that these products failed. This failure was not acknowledged. These serious mistakes have contributed to a measurable decline in public trust in health authorities.[1281]

SOCIETAL IMMUNITY

Messing with complex systems usually leads to detrimental impacts. These might be an effect on health from harmful immune responses or an effect on society from manipulative fear-based messaging. Imagine a societal level immunity that protects us against harmful policies, corruption, erosion of civil liberties and injustice. Various players perform functions similar to different

immune components, identifying threats, amplifying responses and neutralising dangers to the public. Whistleblowers, like phagocytes presenting foreign antigens, identify problematic issues and expose them to the public. The media, acting like cytokines, amplifies this information, raising awareness and rallying public action. Healthy political debate functions like lymph nodes, filtering ideas and rejecting harmful ones. Meanwhile, societal mechanisms like the rule of law, anti-corruption measures, human rights organisations and regulatory frameworks are meant to act as *"antibodies"* designed to prevent tyranny and neutralise threats to civil liberties or ethical governance.

Censorship acts like an immune dysfunction, preventing society from recognising and responding to threats. Prebunking, fact checking and fear propaganda can misdirect rational thinking akin to an autoimmune disease. The result of these is that whistleblowers are silenced, the media is constrained, political opposition lacks access to information and harmful policies are perpetuated while civil rights and ethical frameworks are put at risk. In such an environment institutions like the Judiciary and Parliament act irrationally. Our societal *"immune system"* was overcome and we are now waiting to assess the damage as the last of the media cytokine storm plays out. Even sharing carefully worded facts in 2025 can be met with the same hostility and attacks as in 2020, as if the storm still rages in places. The damage from it is multilayered. It will take time and work to return to a system with a working immune system where whistleblowers are listened to, the media is balanced, the political opposition is informed and plays its role in opposing draconian policies and where civil rights and ethical frameworks are again protected. There is a long way to go.

Conservative member of Parliament Danny Kruger reached the same conclusion when he wrote,

> *Better ministers and advisers may have made better decisions. But fundamentally what failed in 2020–22 wasn't a handful of individuals. It was the system as a whole: the British state, its outer ring of expert advisors, advisory bodies and regulators, and the wider set of supposedly independent institutions, including the media and the pharmaceutical industry, which in this crisis rallied to the State and became to all intents and purposes part of it.*[1282]

None of us has a monopoly on truth but an environment where evidence is shared and assessed fairly depends on us speaking the truth and defending it, no matter the cost. Without this effort, the truth may be lost to the detriment of us all. Only when we work together and listen to each other with respect, can we take steps closer to the truth. Saint Augustine is often credited with the saying,

> *The truth is like a lion. You don't have to defend it; let it loose, and it will defend itself.*

However, truth is not something that survives on its own – it must be actively defended, or it risks being overshadowed by lies. Ironically, the very quote attributed to Augustine is itself false; he never wrote or said it.

Questions arising:
If uptake had not collapsed, would the programme still be running unquestioned today?
What does it say about our system that cost-benefit analysis was an afterthought, not a requirement?
What safeguards must be restored so that truth, dissent and scrutiny cannot be shut down again?

32

THE NEXT PANDEMIC

> *You can avoid reality, but you cannot avoid the consequences of avoiding reality.*
> **Ayn Rand, 1957**[1283]

Swine flu was a forerunner of covid and because no lessons were learned or are being learned we can expect a repeat. The 'next pandemic' has been discussed and even threatened. Here we look at the drivers behind the swine flu episode and covid and the failure of the professional classes to protect the public from these malign influences.

In 2025, Professor Jonathan Van-Tam, the ex-Deputy Chief Medical Officer, who now works for Moderna, said *"a future pandemic is a racing certainty"*.[1284] In 2023, Peter Hotez had already

called covid *"a warmup act"*.[1285] As time went on he escalated his claims and by the end of 2024, made some extraordinary threats,

> *We have another major coronavirus likely brewing in Asia. We've had SARS in 2002, SARS-2 Covid-19 in 2019, and we know these viruses are jumping from bats to people thousands of times a year, but there's still more. We know that we have a big problem with mosquito-transmitted viruses all along the Gulf Coast, where I am here in Texas. We're expecting dengue and possibly Zika virus coming back, or Oropouche virus, maybe even yellow fever, and there's more.* [1286]

In 2024, WHO Director General, Tedros Ghebreyesus, described a need to make plans for 'Disease X',

> *There are things that are unknown that may happen, and anything happening is a matter of when, not if, so we need to have a placeholder for that, for the diseases we don't know…This is a common global interest, and very narrow national interests should not come into the way.*[1287]

The WHO have cultivated increasing power and influence through successive health emergencies, showing a clear direction of travel. This process was well underway by 2009 and the lessons from reports written about the swine flu pandemic have clearly been ignored. By listening to those with most to gain and unfairly shutting down those speaking common sense, we have ended up a long way from rationality.

In 2022, the UK government signed a ten year, £1bn contract with Moderna for the capacity to manufacture 250 million doses of mRNA vaccines in a year.[1288] In order to *"keep it warm"*, ready to switch on mass production, it will produce 100 million doses every year.[1289] What happens to those doses and how did we arrive here?

SWINE FLU AND PHARMACEUTICAL INFLUENCE

In hindsight, the response to swine flu in 2009-2010 was widely criticised, particularly due to the inflated claims about the extent of the threat.[1290] The former head of the WHO's pandemic preparedness, Dr Klaus Stoehr, conceded in 2010 that the declaration of a pandemic was hasty and premature.[1291] In UK, Dame Deirdre Hine's June 2010 Swine Flu report warned against misleading terms like *"reasonable worst case scenario"*, reliance on flawed modelling from *"court astrologers"* and groupthink in SAGE.[1292] It called for proportional responses and better training for officials on scientific advice. The only recommendation that was taken on board 10 years later was that,

> *the behavioural scientists… were not used as effectively as they might have been.*

What a shame.

There was major criticism that pharmaceutical industry influence might have skewed objective decision making.[1293] In January 2010, Dr Richard Schabas, former Ontario Chief Medical Officer of Health, and Dr Neil Rau, an infectious diseases specialist, wrote that the WHO's revised pandemic definition, which did not relate to severity, had sparked an overreaction to H1N1 (the 1918 strain of flu), a virus akin to *"a typical seasonal flu"*.[1294] By mid-November, the outbreak had peaked and waned anyway and they criticized ongoing vaccination efforts, stating,

> *the current risk is exaggerated to justify ongoing futile vaccination efforts directed at the worried well.*

They also noted allegations of pharmaceutical influence raised by others.

In June 2010, the *BMJ* revealed that key WHO advisers had financial ties to pharmaceutical firms set to profit from their guidance.[1295] The European Vaccine Manufacturers (EVM)

which reports to the EU and which includes major vaccine producers such as GlaxoSmithKline, Sanofi Pasteur and Novartis, was found to have worked closely with the WHO in the lead-up to the pandemic declaration. EVM claimed they acted in response to,

> *requests from WHO and governments to have fast access to large quantities of vaccines.*

The *BMJ* investigation raised concerns that some of these same companies had representatives or consultants serving as WHO advisers – raising the possibility that their guidance was not entirely impartial.[1296] The influence alleged came not just from the companies themselves but through these advisory roles, where potential conflicts of interest were not fully disclosed or managed.

Channel Four news presented, Jon Snow, broke the story in 2010 quoting Dr Wolfgang Wodarg, then Chair of the Health Committee of the Parliamentary Assembly of the Council of Europe who said it was,

> *one of the greatest medical scandals of the century.*

He accused,

> *major pharmaceutical firms of organising a campaign of panic and unduly influencing World Health Organization decisions.*[1297]

In the UK, Labour MP Paul Flynn – chair of the All-Party Parliamentary Group on Vaccine Safety – led the charge in exposing the swine flu controversy. He questioned the decisions made, such as the lack of adequate vaccine trials, the distortion of healthcare priorities and the potential influence of pharmaceutical companies on the WHO. He made clear statements about the serious dangers exposed by swine flu none of which were heeded. Flynn spoke about swine flu in parliament in November 2010,

saying even the manufacturers agreed the trials were inadequate,

> *It distorted all the priorities of the health service for a year. The health service gave attention to that rather than to the other things that it should have given attention to. It also involved the use of a vaccine that had not been trialled. The people who say it was not fully trialled are those who made it – GlaxoSmithKline and the other producers. That was a major event, and we might consider, knowing what we know now, how we got into that situation.*[1298]

Paul Flynn presented a report to the European Council in 2010, which accused the WHO of taking a *"highly defensive stance"*.[1299] He complained about exaggeration, conflicts of interest and disproportionate polices and fear saying,

> *Most people, however, expected more dramatic consequences, not least because in spring 2009, the approaching swine flu was repeatedly compared to previous infectious diseases, notably the avian flu and SARS in more recent years, but also the Spanish flu of 1918.*[1300]

He pointed out that political decision making was directed towards,

> *a solution that would protect them from accusations.*[1301]

He said, policy,

> *created a high degree of uncertainty and fear amongst the population, which were not necessarily justified by the evolution of the disease.*[1302]

He talked of a *"concern bias"* where anxiety drives decision making to a greater extent than the disease itself. He argued strongly that vested interests had unduly influenced decision making calling out the lack of disclosure of the members of the WHO emergency committee,

> [how could] *the privacy of experts …prevail over the right of 800 million citizens to be openly and fully informed about*

> *major decisions that might have an impact on their individual health and wellbeing?* [1303]

Paul Flynn died in February 2019, aged 84. The UK's All Party Parliamentary Group on Vaccines where politicians could discuss vaccine concerns died with him and all of this important work to uncover the truth and hold those accountable for their actions has been largely forgotten. Even his own Wikipedia page does not reference his extensive swine flu research or his chairmanship of the parliamentary vaccine safety group.[1304]

The decade after swine flu brought no reform. Instead, the same incentives matured into a new global vaccine-development architecture, with expanding public-private alliances and pre-built rapid response platforms. What had been informal influence in 2009 hardened into a permanent, institutionalised, well-funded system that shaped the default response in 2020.

DIGITAL IDS AND THE NEW MANDARINATE

From 2006-2017 including when the WHO changed the definition of 'pandemic' and oversaw the swine flu episode, the WHO was under the leadership of Margaret Chan, a Hong Kong born Canadian doctor. She was openly backed by Beijing.[1305] After 2006, the WHO began shifting closer to China's interests. That change in tone set the stage for what followed.

China later lobbied hard for Dr Tedros Ghebreyseus to become the first Director General of the WHO in 2017.[1306] With a Dr in front of his name he was trusted even though he had no medical qualification. Aspects to his record that might have reduced trust were not widely shared. Within three months of taking office he announced a strengthened partnership with China. Five months later he appointed Robert Mugabe, the now deceased

Zimbabwean tyrant, as a WHO *"goodwill ambassador"*.[1307] That decision made it clear how dramatically the character of the organisation had changed.

In January 2021, I co-signed an open letter to intelligence services warning about CCP influence and their pushing of lockdowns as an idea.[1308] Lockdown had never appeared in any Western pandemic planning, not even in 1918. The letter highlighted how Western authorities placed blind faith in the CCP: from PCR tests based on theoretical *in silico* genome fragments; to alarmist modelling from Imperial College who advertise themselves as,

> *UK's number one university collaborator with Chinese research institutions*,

to ventilation guidelines taken from early Chinese hospital reports, which, when combined with US and other guidance favouring early intubation, later proved dangerous.[1309,1310] Almost none of it was independently checked.

The factors that led to lockdown are complex and author, Ben Irvine, has explained the nuance.[1311] In Wuhan itself there were long-standing environmental activists who already blamed the authorities for failing to protect the city from pollution. When covid appeared, they seized on it to prove their point that Beijing was reckless about public health, helping to stoke local hysteria. Some of the early clips of people collapsing in the street may well have come from these campaigners and their networks. By mid-January 2020, reports suggested that Wuhan's hospitals were close to paralysis and that medics and teachers were refusing to work. Facing a mutiny of professionals and a city on the edge, Xi Jinping was effectively pushed into authorising a lockdown to restore order locally, not as a national blueprint. Yet he quickly saw how the policy could turn panic into submission. The CCP were paying up to 2 million citizens to post on social media and these accounts flooded Twitter with messages which portrayed

lockdown as a global consensus and suggested that Westerners were begging for similar restrictions.[1312,1313,1314] At that time a satirical song was shared online in Chinese,

After brainwashing, wash your hands and your face.[1315]

The WHO, while not recommending it, commended lockdowns as a *"new standard for outbreak response."* From that moment, pressure on governments came from every direction – above through international agencies and below through domestic unions and fear-driven activism. Western drivers included the broader global-health network pushing hard for a vaccine-centred strategy and, in the UK, threats from the National Education Union, which were decisive. Incentives aligned and policy spread faster than the virus.

During that period Bruce Aylward, a Canadian epidemiologist was serving as a senior WHO adviser and its liaison in China. When a reporter asked him on live video if the WHO would discuss Taiwan's independent response to covid, he pretended not to hear the question, terminated the call, and then rejoined only to say,

Next question, please, we've already talked about China.[1316]

It was one of the defining moments of 2020. A WHO representative publicly refused to acknowledge Taiwan rather than offend Beijing. From 2021, the claimed flat plateau in cumulative covid deaths in China became a more organic looking tiny increase. Our World in Data, the Oxford based data repository referenced by governments and media alike, had folded Taiwan's data into the Chinese dataset, erasing Taiwan a second time.[1317]

The CCP Filter

Vaccine design was based on the first genetic sequence of *SARS-CoV-2* released by Beijing in January 2020. That single unverified

file became the basis for Western vaccine design. The same Chinese sequence was adopted as the template for global testing. The fact that later sequencing aligned with it does not mean the Chinese sequence was an exact representation of the viral genome – alignment means it approximates the sequence not that it was exactly the same. None of the subsequent sequences matched it exactly. It should never have been accepted uncritically – especially given that China's 2011 submission to the UN Biological Weapons Convention had openly discussed how to *"increase the destructiveness of biological weapons"*. It said,

> *Foreign genes or viruses can be introduced into the target population asymptomatically by means of gene-therapy vectors, enabling a biological weapon attack to be mounted covertly.*[1318]

Only naivety and gullibility or else forgetfulness can explain how CCP was trusted given that history on record.

Inside China, every scientific paper on covid was screened and edited by the Ministry of Science before release.[1319] Chinese papers flooded the international medical literature and were quoted as *"The Science"*. There will always be honourable scientists in China, but under a political state their work is never independent. The volume and the detail to which every topic was covered should have triggered scepticism. Instead, the world bowed.

Building the architecture before covid

Long before the public heard the phrase 'vaccine passport', the groundwork had already been laid. In the UK, Tony Blair's government had spent years constructing a national digital identity scheme under the 2006 Identity Cards Act, including biometric databases and compulsory ID enrolment.[1320] After the 2010 election, the incoming Cameron–Clegg coalition judged the system so dangerous that it ordered the entire database to be physically destroyed in 2012 – cutting up the drives holding the

15,000 identities already collected.[1321] It was widely understood as a point-of-principle rejection of a centralised identity regime.[1322]

Blair later revived his campaign for digital identity, insisting throughout the 2020s that a national digital ID was *"inevitable"* and should be adopted.[1323] By 2019, the European Commission called for an *"EU citizen digital vaccination card"* designed to be compatible across borders and accessible internationally.[1324] This was the first step towards a globalised immunisation-identity system.

By early 2020, the WHO had moved from aspiration to construction. It created a *Smart Vaccination Certificate Working Group*, with an open call for experts that began before any covid vaccine had been authorised and closed on 14 December 2020.[1325] The infrastructure was being built before the product existed.

In August 2021, the WHO released a 99-page technical specification for *Digital Documentation of COVID-19 Certificates*.[1326] It was longer than any WHO publication on the disease itself. Buried in the text was the real purpose: covid was described merely as the short-term rationale for developing *"a sustainable solution"* to be used for,

> *other existing health-system challenges and future pandemics.*

In other words, this was not an emergency response; it was a permanent system.

The language revealed even more. People were not *"patients"*, *"citizens"*, or *"the public"*, but *"data subjects"* and the dystopian *"Subjects of Care"*. The document detailed how a digital pass could be revoked and how authorities should verify that revocation, yet offered no meaningful mechanism for a person to regain control of their identity if stolen or misused. Power flowed only one way.

Although the WHO insisted there should be *"no discrimination"* between paper or digital passes, the entire architecture enabled discrimination by design. And rather than nations simply recognising each other's records, the WHO proposed an internationally linked system under its own oversight. The global identity backbone was constructed quietly, before most people had any idea what was coming.

Increased control

The Chinese Communist Party's influence within the WHO reached new heights in May 2022, when Beijing secured a seat on the organisation's executive board. Journalist Ross Clark said WHO had lost all credibility, writing,

> *America and Canada apart,* [the board] *is stuffed with small countries, many with lousy human rights records, which will not dare to challenge China or which will not have the political clout to do so.*[1327]

He was not wrong. The WHO is no longer a neutral health body; it has become a political instrument.

In 2023, the WHO proposed a new Pandemic Treaty and amendments to the International Health Regulations. Although later watered down, these show an intent for the WHO having unprecedented power and it seems reasonable to be concerned that there will be mission creep over time. Even if these changes could be attributed to bureaucratic expansionism the end result is much the same.

The original wording included requiring countries to allocate at least 5 percent of health budgets and an unspecified percentage of GDP to pandemic preparedness.[1328,1329] They wanted a global database of all travellers including vaccination certificates, passenger locator forms and biometric identifiers. The morphing

of the vaccine passport infrastructure into digital ID plans globally will enable that. They also sought control over global supply chains and pandemic stockpiling, regular pandemic simulations, deployment of a global health workforce and mandatory sharing of resources and data. The initial proposal even suggested removing a line saying the regulations should be based on,

> *full respect for the dignity, human rights and fundamental freedoms of persons.*[1330]

Another lockdown could be announced based only on a potential threat. If it were announced, people might well dust off their banana-bread recipes and show the young how it is done, but it would still be entirely unjustified. Lockdowns cannot slow the spread of any aerosolised virus, all of which can travel long distances in the night air. Faeco-oral transmission, by contrast, spreads slowly and is already controlled by isolating those who are visibly unwell. Claimed severity does not change the basic biology of how these pathogens spread, nor our inability to control them through lockdowns.

These proposed measures would have pushed us closer to the WHO being able to impose restrictions on national and individual liberties during emergencies. Even if sovereignty was not officially over turned and even if politicians wanted to take a different course, they would struggle to persuade civil servants and the public that they should ignore the WHO.

Even worse, the WHO constitution itself was written during a period of pathological optimism and contains no real safeguards against dictatorial behaviour. The Director General already has absolute discretion to declare a *"pandemic"* or a *"potential pandemic"*. Tedros tested that power in 2022, when he overruled the Emergency Committee that he had installed, to declare a Monkey Pox Public Health Emergency of International Concern or

PHEIC (pronounced 'fake'). This is the highest alert before declaring a pandemic. He said about the committee's vote,

> *Nine and six are very, very close. Since the role of the committee is to advise, I then had to act as a tiebreaker.*[1331]

This is what unaccountable power looks like.

In July 2022, the WHO appointed Susan Michie. She is the British behavioural psychologist and long-time communist who helped run the UK's covid *"nudge"* project responsible for the fear strategy.[1332] She will chair their Technical Advisory Group on Behavioural Insights and Sciences for Health – presumably population behavioural manipulation is now part of WHO strategy.

Then under its One-Health banner the WHO claimed authority over human, animal and environmental health with the excuse that they want to prevent zoonotic disease. Sacrificial culling of animals can then be justified in the name of controlling disease outbreaks.[1333] Millions of poultry have been culled in the latest avian flu panic, which is particularly galling when not only is the virus airborne but the wild bird population cannot be culled. The illogical and unscientific nature of these measures make them appear like ritualistic sacrifices to appease *"Pandemic Gods"*. The breadth of control the WHO seeks is astonishing.

Strategic Capture and Western compliance

In October 2025, Tim Shipman reported in *The Spectator* how Beijing had been allowed to purchase a company operating a secure hub used for internal government communications.[1334] Data was breached for months. When they were briefed, both Boris Johnson and Rishi Sunak were reportedly *"open-mouthed in amazement."* No one was dismissed.